

ASPIRIN, VIAGRA, AND COFFINS

THE IMPOLITE MANUAL OF
INEVITABLE OPPORTUNITIES

AN EXPERIMENT IN RESEARCH AND WRITING
WITH ARTIFICIAL INTELLIGENCE

ASPIRIN, VIAGRA, AND COFFINS

The Impolite Manual of Inevitable Opportunities

AN EXPERIMENT IN RESEARCH AND WRITING
WITH ARTIFICIAL INTELLIGENCE

Copyright © [2026] Axiologic Research

All rights reserved.

KDP publishing rights held by Outfinity SRL (Iași, Romania).

Available for free on Axiologic website (www.axiologic.net).

Author's Note

This work was created under the umbrella of Axiologic Research as part of two interconnected research programs, one dedicated to Artificial Intelligence and the other to Outfinitism, both led by Sînică Alboaie. To explore the details of how these two programs intersect and build upon each other, we invite readers to visit the Outfinity Venture Studio page at www.outfinity.ch.

While Artificial Intelligence language models were used to assist with text generation, the foundational philosophy, thematic structure, and analytical approach derive exclusively from the author's decades of dedicated study of social phenomena, social technologies, and genuine hard technologies resulting from various European and self funded research projects. Recently, within the Achilles research project, we have been deeply studying AI generated literature and its automated verification using neuro symbolic approaches; all the free books made available to the public are part of the suite of works we use to test our latest technologies.

CONTENTS

NOTE ON THE EXPERIMENT.....	6
Why this book hides its sadness behind a bad-mannered smile.....	6
PROLOGUE.....	9
The market does not love people, but it studies their reflexes impeccably.....	9
CHAPTER 1.....	13
Before the Factory: Paradise Had No Forms, but It Had Fever, Shame, and Very Attentive Gods.....	13
CHAPTER 2.....	18
The Industrial Revolution: When the Clock Became the Boss and Humanity Earned the Privilege of Being Late.....	18
CHAPTER 3.....	22
The Century of Comfort: We Removed Many Pains and Invented Enough Desires Not to Notice.....	22
CHAPTER 4.....	26
Diagnosing Modernity: A Paleolithic Body, Industrial Institutions, and a Mirror That Refreshes.....	26
CHAPTER 5.....	30
The Aspirin Signal: When the Problem Runs Its Own Product Demo.....	30
CHAPTER 6.....	35
The Viagra Signal: Immediate Effect, Discreet Shame, and Margins That Prefer Artificial Light.....	35
CHAPTER 7.....	39
The Coffin Signal: The Safest Market Is the One Nobody Leaves Alive.....	39
CHAPTER 8.....	44
The Phone That Compares Us: Social Media as an Exception Factory and Microdose Pharmacy.....	44
CHAPTER 9.....	49
The AI That Listens: Therapist, Companion, Secretary, and Cognitive Pimp.....	49
CHAPTER 10.....	54
An Atlas of Unpleasant Industries: Where Smell, Shame, and Bureaucracy Protect the Margin.....	54
CHAPTER 11.....	59

The Great Indecent Question: Is Psychological Distress the Next Megamarket—or Merely the Next Colony?.....	59
CHAPTER 12.....	64
The Researcher, the Futurist, and the Person with a Ring Light: Three Ways to Predict Without Paying for Error.....	64
CHAPTER 13.....	69
How to Sell the Antidote Without Becoming the Poison: An Ethics Manual for People Who Still Have Payroll.....	69
CHAPTER 14.....	73
Escaping Modernity Without Losing the Dentist: An Architecture for Lives, Organizations, and Markets That Do Not Demand Total Nervous Sacrifice.....	73
EPILOGUE.....	78
When the sarcasm ends, the bill remains.....	78
SELECTED BIBLIOGRAPHY AND NOTES FOR ORIENTATION.....	80

NOTE ON THE EXPERIMENT

Why this book hides its sadness behind a bad-mannered smile

This book is an explicit experiment in writing with artificial intelligence. Our goal was to use AI to distill valuable signals for investors and entrepreneurs by analyzing the real pains of the modern world through a caricatural lens. Although we requested a deliberately sardonic tone, the AI failed, producing a relatively polite book instead. Ultimately, our attempt to test the AI's understanding of sardonic humor was unsuccessful. Not because anxiety, loneliness, depression, addiction, exhaustion, sexual shame, cognitive decline, or death are funny. They are not. Sarcasm is used precisely because the cargo is difficult to move. It succeeds to be a bit ironic and uses some strong words to attract attention in a caricatural way. A little ironic distance sometimes lets the reader look longer at a truth that would otherwise be rejected after two paragraphs. Laughter does not close the wound; it merely keeps the page from being closed too soon.

This book does something suspicious: it treats modern suffering simultaneously as human tragedy, research object, and market signal. Ordinarily, the three are kept in separate rooms. Tragedy receives candles and speeches. Research receives footnotes and sentences afraid to breathe. The market receives a pitch deck in which nobody dies, nobody cries, and every problem is “an addressable opportunity.” Here they sit at the same table, and we watch to see who asks for the bill first.

Research, synthesis, argument design, and a substantial share of the drafting were assisted by a language model. This is not concealed behind an author photograph of someone gazing melancholically through a window. It becomes one of the book's subjects. A statistical system writes about statistical systems that are beginning to console us, assess us, hire us, fire us, seduce us, and, in the least fortunate cases, confirm our delusions with impeccable manners. The experiment is not meant to prove that a machine “understands” human tragedy. It asks whether a machine can organize evidence and sharpen questions until humans can no longer hide comfortably behind jargon.

The central thesis is simple and unpleasant. Modernity did not invent psychological suffering. It would be a remarkable form of historical vanity to

imagine that early humans discovered fire, agriculture, war, and gods but needed us to discover despair. Anthropological and historical evidence describes suicide, psychosis-like states, pathological grief, fear, shame, and alienation long before the factory and the smartphone. Modernity did, however, add something distinctive: it industrialized exposure to chronic stressors, standardized social comparison, more often detached individuals from stable communities, converted time into a production unit, and turned attention into advertising inventory. Then, with characteristic efficiency, it built industries that sell relief from the effects of industries that sell stimulation.

This is where the indecent entrepreneurial question appears. When pain is persistent, urgent, and immediately legible to the customer, we have the Aspirin signal. When the benefit is immediate, intimate, difficult to discuss, and entangled with status or shame, we have the Viagra signal. When demand is inevitable, regulated, fragmented, and emotionally impossible to postpone, we have the Coffin signal. These are not products but lenses. They reveal markets in which the customer does not need an essay explaining “why now.” The body, shame, or mortality has already written the investment memorandum.

The framework can deteriorate quickly into business-school cynicism: vulnerable people reduced to acquisition cost, loneliness converted into retention, compulsion renamed engagement, and existential crisis wrapped in a premium subscription. The book refuses to let that deterioration pass unnoticed. An entrepreneurial opportunity is not automatically a moral permission slip. Some businesses reduce suffering. Others manage it just well enough to keep it from disappearing. The most profitable models may be precisely those that promise the antidote while requiring the poison to remain in circulation.

Research therefore appears here not as academic upholstery but as a brake. The World Health Organization estimated that roughly one person in eight lived with a mental disorder in 2019 and described care systems as inadequate and radically unequal (WHO, 2022). The 2025 report of the WHO Commission on Social Connection estimated that about one person in six experiences loneliness, with particularly high prevalence among adolescents and young adults (WHO, 2025). In 2026, the International Labour Organization estimated that occupational psychosocial risks are associated with more than 840,000 deaths a year, nearly 45 million disability-adjusted life years, and economic losses equivalent to 1.37 percent of global GDP (ILO, 2026). These figures are not a customer list. They are evidence that the design of institutions, work, and technology can become pathogenic infrastructure.

The voice will move intentionally between analysis and satire. When a paragraph sounds too elegant to be moral, the next should check its pockets. When a prediction sounds inevitable, we will ask who profits if we believe it. When an influencer announces an “epidemic” caused by a single app, we will look for the meta-analysis. When a researcher says the average effect is small, we will ask whether the average conceals the minority for whom the effect is devastating. When an entrepreneur promises to democratize access, we will check whether only the automated version is being democratized while human care remains reserved for people who can afford to be treated as human.

The reader will not receive a list of startup ideas that can be copied over a weekend and pitched on Monday morning. The reader will receive a map of discomfort, inevitability, and neglected infrastructure. It includes sexual health, death and grief, sleep, loneliness, care coordination, behavioral addictions, chronic pain, medical and legal bureaucracy, security, waste, and maintenance. Some fields look glamorous only to investors who have never done the work themselves. Others are profoundly boring, which means they have been ignored long enough for modest technology, competently deployed, to look like magic.

The book’s final test is this: can we keep the advantages of modernity without paying its full psychological price? The answer is unlikely to be an app that reminds us to breathe between two notifications engineered to stop our breathing. It will be a combination of better-aligned products, organizations that remove stressors at the source, communities that rebuild reciprocity, and policies that make the exploitation of vulnerability less profitable. Entrepreneurship has a real role—perhaps an enormous one—but only if it learns the difference between monetizing pain and removing the reason it hurts.

This is not medical, legal, or financial advice. It is an essay in research and strategy written in a literary register. Empirical claims are accompanied by references, and where the evidence is mixed, the text tries to preserve the ambiguity even when sarcasm would prefer a more photogenic culprit.

PROLOGUE

The market does not love people, but it studies their reflexes impeccably

In every age, people have found a way to turn the inevitable into an occupation. Someone dug graves before anyone invented branding. Someone eased pain before aspirin acquired packaging, patents, and a slogan. Someone brokered sex, shame, fertility, seduction, and reputation before platforms discovered that loneliness converts extremely well. Civilization can be read as a long succession of attempts to package the body's needs and the mind's fears as billable services.

When entrepreneurs speak about opportunity, they prefer luminous words: "transformation," "empowerment," "community," "wellness," "democratization." These words have the advantage of being odorless. They do not evoke blood, sweat, libido, incontinence, bankruptcy, divorce, cognitive decline, or corpses. Venture capital has exquisitely refined olfactory sensitivities. It likes enormous problems, but only after someone has sterilized them in PowerPoint.

Yet the most durable businesses are not always those promising a poetic future. They are often the ones that intervene when the customer cannot negotiate with reality. The pipe has burst. The father has died. The password was stolen. The pain will not stop. Sleep will not arrive. The erection will not. The child no longer leaves the bedroom. The mother can no longer live alone. The company does not understand the regulation, and the fine has a deadline. These markets possess an obscene strategic advantage: the problem does the marketing.

Aspirin is the first signal. The customer knows it hurts. Nobody needs to educate the customer about the category; the task is to prove that the product works, is safe, and is available now. In economic language, the cost of explaining the problem is low, urgency is high, and feedback is fast. In human language, the person wants it to stop. Aspirin markets appear wherever friction is repetitive, obvious, and measurable: pain, insomnia, bureaucracy, fraud, waiting, failed coordination, and errors that recur with the punctuality of a public administration.

Viagra is the second signal. Here the problem is personal, burdened by shame or status, and attached to an immediate reward. The customer may avoid the conversation, but not the need. Viagra markets combine discretion, rapid results,

permanent availability, and a willingness to pay that consumer interviews underestimate because people lie more politely when the question is intimate. Sexual health is the obvious example, but the structure repeats in fertility, menopause, hair loss, performance anxiety, reputation, dating, infidelity verification, odor control, incontinence, and nearly every problem for which the customer clears the browser history.

The coffin is the third signal. Demand is inevitable, the decision window is short, the family is vulnerable, the process is fragmented, and the cost of error is emotional as well as legal. Funeral services are not the only coffin market. Digital estates, will administration, account transfer, palliative-care coordination, postmortem data security, company closure, property clearance, and the management of personal effects all belong to the same economy of inevitability. Wherever everyone eventually arrives but nobody wants to research in advance, the competent intermediary can become priest, logistics operator, and local monopoly in a single afternoon.

TABLE 1. THE THREE ENTREPRENEURIAL SIGNALS

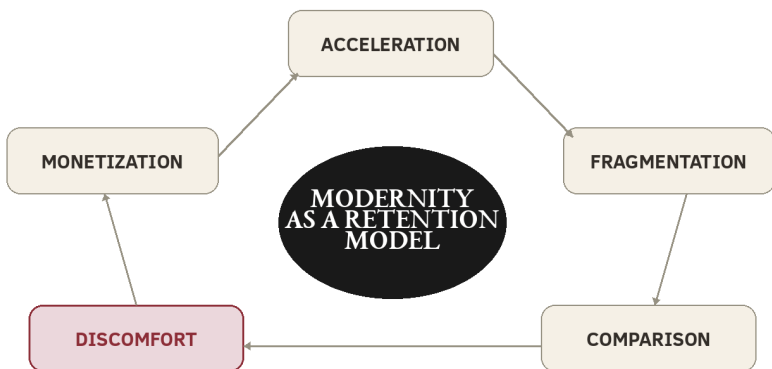
SIGNAL	SIGNAL AND IMPLICATION
Aspirin	Self-evident pain; high urgency; rapid feedback; value is the disappearance of the symptom.
Viagra	Immediate benefit; intimacy and shame; discretion; willingness to pay; recurrence.
Coffin	Inevitable demand; decision under pressure; information asymmetry; trust and coordination.

The book argues that the next large family of opportunities lies where the three signals overlap: recognizable psychological distress, immediate benefit, and the inevitable problems of a technologically mediated life. The modern world produces fatigue, comparison, uncertainty, and loneliness not merely as isolated accidents but as side effects of systems optimized for something else. The platform optimizes time spent, not peace of mind. The employer optimizes output, not biographical coherence. The city optimizes density and mobility, not belonging. The housing market optimizes yield, not proximity to friends. Artificial intelligence optimizes the relevance of the answer, not the maturation of the person asking.

Saying that these systems “create” every problem would be too simple and, worse, commercially useful. A single villain is a gift to consultants. Reality is less

cooperative. Average associations between digital-technology use and adolescent well-being are often small, and findings vary with measurement, population, and design (Orben & Przybylski, 2019; Valkenburg, Meier, & Beyens, 2022). At the same time, problematic or addictive patterns, nighttime exposure, social comparison, cyberbullying, and pre-existing vulnerabilities can produce much larger risks for some people. The statistical average is an excellent diplomat: it reconciles the general population with the minority that is collapsing, then leaves before the damages are due.

The real opportunity is not to “solve mental health” inside an app with a purple gradient. It is to identify where modernity generates repeatable friction and where an intervention can change the environment rather than merely adjust the individual’s reaction. In some cases, the product will be clinical and regulated. In others, it will be social infrastructure: clubs, meeting places, care services, intergenerational housing, or return-to-work programs. Elsewhere, it will be boring software that reduces bureaucracy, coordinates appointments, or detects risk without pretending to be the perfect friend.



When relief is paid for by the persistence of the problem, the loop becomes a business model.

Figure 1. The commercial loop of modernity. Acceleration produces frictions that can be sold back as relief.

Here the moral line appears. A good business can earn money by reducing a problem. A business dependent on the problem earns more when the problem persists. The first has an interest in curing you or making you autonomous. The

second has an interest in stabilizing you in functional insufficiency: bad enough to return, well enough to pay. The modern attention industry has perfected this state. It does not want you happy; happy people forget their phones in another room. It does not want you completely destroyed either; completely destroyed people stop clicking. It wants you mildly anxious, mildly lonely, mildly outraged, and always within reach of one more dose.

The chapters ahead compare regimes of stress from hunter-gatherer societies and agrarian life through the ancient city, the industrial factory, the corporate office, the social network, and the AI companion. This is not an Olympics of suffering. We will not declare that medieval famine was “more authentic” than a panic attack in an open-plan office. We will ask what kind of threat dominated, how long it lasted, who shared it with you, what social meaning it received, and who could profit from it.

We will then examine unpleasant industries. Not to celebrate exploitation, but because inefficiency, opacity, and abuse tend to accumulate wherever people prefer not to look. Death, sex, old age, addiction, pain, waste, and fraud do not disappear because a technology conference selected a more optimistic title. They are also fields in which trust, integration, and careful design can create real value, even when the product will never appear in a palm-tree selfie.

Finally, we will attempt an escape without romance. Not a return to the cave, where nature was the dentist and life expectancy had a sense of humor. Not a theatrical seven-day detox followed by reinstalling every app at the airport. Instead, a selective modernity: keep antibiotics, access to knowledge, productivity, and the freedom to leave oppressive communities; refuse to accept that notifications, surveillance, work without edges, and algorithmically assisted loneliness are the unavoidable price of progress.

The market does not love people. But it studies their reflexes impeccably. The question is whether the entrepreneur can use that knowledge to free a person from the reflex rather than merely make the reflex more predictable.

CHAPTER 1

Before the Factory: Paradise Had No Forms, but It Had Fever, Shame, and Very Attentive Gods

Every critique of modernity risks beginning with an imaginary photograph: a small group gathered around a fire, perfectly integrated into community, eating local food, sleeping by the sun, and entirely unaware of LinkedIn. The picture is seductive. The light is warm, there is no ambulance, and nobody appears to die in childbirth. It is, in essence, marketing material for a past that cannot demand a right of reply.

Hunter-gatherer societies and other small communities did, in many settings, provide things modernity has made scarce: repeated social contact, recognizable roles, exposure to nature, physical movement, circadian alignment, and a direct relationship between effort and result. They also offered high mortality, intergroup violence, vulnerability to hunger and infection, little privacy, severe constraints on individual choice, and spiritual or punitive responses to experiences we might now call mental disorders. A close community can be medicine. It can also be a panopticon without an unsubscribe button.

The anthropology of suicide is a useful correction to lazy nostalgia. Michel Tousignant's review of small-scale societies found that suicide existed before radical acculturation and that the relationship between social change and suicidal behavior is complex—not a linear tale of urbanization followed by alienation (Tousignant, 1998). In some communities, the act could function as a repair of honor or as a social message inside a family conflict. Smith and Hackathorn's comparative study, based on a small sample of societies then described as "primitive" or peasant, suggested associations between suicide prevalence, stable agriculture, extreme norms of emotional expression, and a cultural emphasis on pride and shame (Smith & Hackathorn, 1982). The period's categories and methods were limited, but the durable conclusion is plain: suffering did not wait for platform capitalism before learning to weaponize social status.

A small community reduces anonymity and can increase support. It also makes failure visible and reputation difficult to reset. In a modern city, you can change neighborhood, profession, partner, public identity, and—budget permitting—your face. In a group of fifty people, your biography is shared infrastructure. Everyone knows who refused the alliance, who withheld food, and who cried at the wrong time. The modern freedom to reinvent oneself is, among other things, the right to escape social memory.

Agriculture changed the psychological regime. Sedentary life enabled surplus, property, hierarchy, taxation, and organized war. It also enabled temples, writing, medicine, monumental art, and that admirable invention known as the civil servant. Humans gained seasonal predictability and became vulnerable to harvests, landlords, debts, and rules issued by people they could not see. The acute stress of hunting increasingly coexisted with the chronic stress of obligation. Nature could kill without explanation; administration added the explanation in triplicate.

The ancient city amplified density, specialization, and comparison. Athens, Rome, Alexandria, and Chang'an were not merely centers of learning and trade. They were machines of stratification. Wealth became visible, status performative, and citizenship a technology of inclusion and exclusion. Greco-Roman texts contain descriptions that, with every retrospective caveat, resemble psychosis, melancholia, mania, delusion, and misanthropy. A systematic review of schizophrenia-like descriptions in Greek and Roman literature found suggestive material, although retrospective diagnosis remains methodologically precarious (Evans, McGrath, & Milns, 2003). Ancient sources mainly show that pathological experiences were translated into the cultural, moral, and religious categories available to the period. The absence of our clinical vocabulary is not the absence of suffering.

That difference in vocabulary matters. Without a diagnostic manual, suffering does not disappear; it receives another explanation. It may become possession, punishment, imbalance of the humors, moral weakness, divine inspiration, or contamination. Modern medicine reduced a great deal of cruelty by recognizing illness, but it also created the temptation to turn every social mismatch into an individual label. In the past, a community might attribute suffering to the gods. Today, an organization may attribute it to an employee's insufficient resilience. The gods and Human Resources occasionally share a talent for avoiding structural responsibility.

Medieval Europe, the medieval Islamic world, Asian empires, and African or American societies cannot be compressed into a single psychological experience.

They contained institutions of care, sophisticated medical traditions, community practices, and long periods of stability. They also contained war, epidemic disease, famine, servitude, infant mortality, and constant familiarity with death. A premodern person was not necessarily more anxious in the clinical sense, but lived in a world where catastrophe had a high material probability and a ready cosmological explanation. Meaning does not remove pain, but it changes its form. A death inside a ritualized community can be devastating and still be collectively held. Burnout inside a corporation may be less immediately lethal and still be experienced as private failure while colleagues send heart reactions on the internal platform.

Comparing eras requires three precautions. The first is data survival. We have more diagnoses now because we have more clinicians, instruments, reporting systems, and people who live long enough to develop or report chronic conditions. The second is definitional change. Ancient “melancholia” and contemporary major depression overlap imperfectly. The third is nostalgic selection. We notice the solidarity of small communities and forget sanctions, violence, and the inability to leave. We notice modern freedom and forget the psychological cost of permanent choice.

TABLE 2. WHAT EACH SOCIAL REGIME GIVES US—AND WHAT IT CHARGES

REGIME	SIGNAL AND IMPLICATION
Hunter-gatherer	Acute, collective threats and intense belonging; mortality, scarcity, and constraint.
Agrarian	Stability and surplus; hierarchy, property, shame, and chronic obligations.
Ancient city	Specialization and culture; stratification, density, and public reputation.
Industrial	Wages and productivity; disciplined time, reduced autonomy, and migration.
Mass media	Comfort and consumption; standardized desire and comparison.
Networked	Access and connection; infinite feed, permanent evaluation, and fragmented sleep.
AI-mediated	Assistance and personalization; delegated judgment and simulated relationships.

Even with those cautions, comparison reveals an important transformation: a shift from threats that were predominantly episodic and shared toward threats that are chronic, abstract, and individualized. Famine was collective, even when its distribution was unequal. A performance review is personal, even when the algorithm applies it to ten thousand people. A raid came from outside and eventually ended. Email arrives from every time zone and has a “follow-up.” Traditional danger mobilized the body for action. Modern danger often mobilizes the body for action it cannot perform: you cannot punch an interest rate, outrun an indexed reputation, or negotiate with a feed that has learned exactly what irritates you.

This does not mean the nervous system is a defective relic. It means modern institutions exploit adaptive mechanisms in contexts for which they were never calibrated. Social comparison helps a person in a small group understand rank and norms. In a network of billions, it becomes a conveyor belt of exceptions: bodies, careers, holidays, houses, and relationships selected precisely because they are not normal. Vigilance helps when a rustle in the bushes might be a predator. In the news feed, every alert can be a predator, and the device has commercial reasons not to clarify that most predators are not in your room.

For the entrepreneur, the past offers a lesson that fits poorly inside an accelerator program. Basic psychological needs are not new, and products serving them need not imitate old rituals as décor. We do not need an app with an animated campfire. We need infrastructure that restores the functional properties of community: reciprocity, repetition, accountability, embodied contact, common purpose, and low participation cost. Local clubs, intergenerational care, community workspaces, cooperatives, mentoring programs, and services that reduce isolation have potential precisely because they sell something old in a society that has made it scarce.

There is, however, a warning. “Community” has become a word companies apply to any group of customers receiving notifications. A real community may ask for sacrifice, notice absence, and offer help without requiring an upgrade. An audience is not a community. A user base is not a community. A comment section in which optimized identities quarrel is not the village square, although it may reproduce public shame perfectly while failing to provide bread.

The market of the future will rediscover belonging services. Some will be authentic; others will sell simulation. The difference can be measured by what remains when the subscription ends. If the person leaves with friends, skills, routines, and a position in a group, the product created social capital. If the person

leaves with only a streak, a badge, and anxiety about losing access, the product created decorated captivity.

We do not have to choose between the cave and the platform. We can keep modern medicine, individual rights, mobility, and knowledge while deliberately rebuilding what preindustrial settings often supplied without an app: repeated relationships, common rhythms, and the sense that one's effort is witnessed by people who will still be there tomorrow. This is the first escape from modernity—not flight into the past, but recovery of functions progress treated as acceptable losses.

What changes is not the existence of suffering, but its form and distribution.

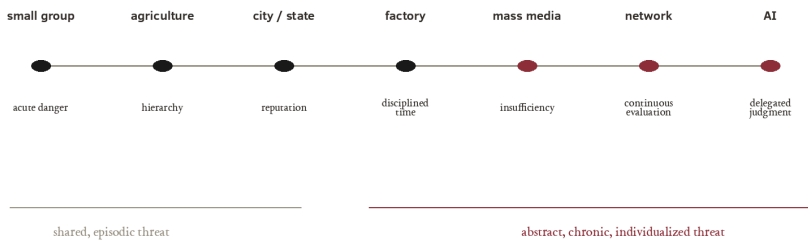


Figure 2. The historical shift in dominant stress: from acute, shared threats to abstract, chronic, individualized exposure.

CHAPTER 2

The Industrial Revolution: When the Clock Became the Boss and Humanity Earned the Privilege of Being Late

The Industrial Revolution is often presented as a story about machines. In psychological terms, the most important machine may have been the clock. The steam engine increased physical power; standardized time reorganized life. E. P. Thompson described the rise of industrial time discipline: the transition from labor organized by task and natural rhythm to labor measured in hours, supervised, and sanctioned (Thompson, 1967). A person no longer finished when the job was done. The job was done when the siren announced that the person's time could be temporarily returned.

That transformation produced goods, cities, wages, and eventually enormous gains in life expectancy and comfort. It also produced a radical separation among workplace, home, family, and community. Exploitation was real in domestic and agrarian economies, but activity was often embedded in relationships and seasons. The factory extracted work from context and made it comparable. Once hours and output can be measured, the person becomes a variable. Scientific management turned that intuition into a discipline; much later, software gave it a dashboard.

Industrial cities concentrated populations, pollution, disease, accidents, alcohol, inequality, and opportunity. In 1845, Friedrich Engels described the condition of the English working class with a mixture of social observation and political indignation. It was not modern epidemiology, but it captured a mechanism: density without social and sanitary infrastructure can turn physical proximity into moral isolation. You can live wall to wall with hundreds of people and still have nobody able to lend you time.

Émile Durkheim linked suicide to social integration and regulation, arguing that modernization can produce anomie—the weakening of norms and limits that organize desire (Durkheim, 1897). The theory has been criticized, refined, and occasionally caricatured, but its question remains central: what happens when individual freedom grows faster than institutions' capacity to provide belonging and

meaning? A historical study of Finland from 1800 to 1984 found an association between urbanization and rising suicide rates, stronger in the nineteenth century and weaker later, suggesting both the cost of transition and the possibility of institutional adaptation (Stack, 1993). Modernity can injure, but it can also learn. Unfortunately, it often begins learning only after the accounting becomes embarrassing.

Urbanization is neither a single cause nor a verdict. Vassos and colleagues estimated that schizophrenia risk in the most urban settings was about 2.37 times that in the most rural settings (Vassos et al., 2012). Possible mechanisms include social adversity, migration, discrimination, isolation amid density, environmental exposures, and selection. This does not mean that cities automatically “produce” psychosis or that villages are clinically blessed. It means that social and physical environments enter the causal picture, and urban design is not merely a matter of traffic and façades.

Industrialization also changed the family. Paid work was separated from domestic work, and the economic value of care became easier to ignore precisely because it received no wage. The ideal industrial man was supposed to be punctual, disciplined, durable, and detached. The ideal domestic woman was supposed to absorb the consequences without issuing an invoice. When those roles failed, the problem was attributed to character, nerves, or morality. Early modernity was highly skilled at producing structural conditions and then diagnosing the reactions one individual at a time.

At the same time, modern psychiatry, asylums, statistics, and public medicine made suffering more visible. Visibility can resemble growth. Build institutions and categories, and you find cases that were previously concealed in the family, monastery, prison, street, or cemetery. Crude historical comparisons are therefore unreliable. We cannot compare the number of diagnoses in 1850 and 2026 and declare that the mind deteriorated by an exact factor. We can track environmental changes and plausible mechanisms: externally controlled time, reduced autonomy, insecurity, migration, broken networks, and visible inequality.

Here appears the first major entrepreneurial lesson of industrialization. When a technology reorganizes production, it does not create only efficiency gains; it creates secondary industries that repair the effects of reorganization. The factory created demand for urban housing, transport, insurance, occupational medicine, mass recreation, industrial alcohol, and labor movements. Some reduced harm. Others

anesthetized the worker just enough for the next shift. Aspirin and the tavern are strategic relatives: both understand that recurring pain produces retention.

The pattern repeats today. Digital work removes commuting for some and dissolves the boundary of the working day for many. Automation reduces tasks and increases uncertainty. Artificial intelligence can increase productivity while turning every employee into a permanent reskilling project. Research on automation identifies psychosocial mechanisms including job insecurity, work intensification, loss of control, monotony—or, in the other direction, development opportunities and relief from repetitive tasks. Technology does not have one effect. It distributes power. Mental health often follows the distribution.

A 2025 study of occupational exposure to automation reported a higher probability of anxiety or depression, partly mediated by job insecurity and expectations of changing skill requirements (Blasco, Rochut, & Rouland, 2025). This is not proof that a robot causes depression by existing. It is evidence that anticipated loss of control can become a stressor before the machine is installed. Modern markets have discovered that the future can be monetized twice: first as a promise to investors, then as anxiety for employees.

In 2026, the International Labour Organization published global estimates of the burden associated with psychosocial risks at work, including long hours, insecurity, high demand with low control, effort–reward imbalance, and harassment (ILO, 2026). Its central recommendation is not merely that employees learn to meditate more competently. Risks should be prevented through organizational design, participation, autonomy, limits, and management. The World Health Organization draws the same distinction: individual interventions can help, but employers must alter the conditions generating the risk (WHO, 2022b).

For the market, this distinction is explosive. The corporate-wellness industry prefers individual interventions because they can be purchased without changing the structure. A mindfulness app costs less than hiring three more people. A resilience webinar is easier than reducing impossible targets. A “mental health day” photographs well; actual control over the schedule requires negotiation. A company can manufacture stress on Monday and sponsor conscious breathing on Thursday, earning two press releases and preserving the same margin.

A genuinely valuable business will not sell only individual calm. It will measure and reduce organizational causes. Work-design tools, capacity planning, overload detection, fair rotation, anti-harassment systems, graduated return to work, and audits of algorithmic surveillance could become major markets. But there is a trap:

the same platform promising to detect burnout can become an emotional-surveillance system. If the employer can see who is “at risk,” the question is not only how help will be offered. It is also who will be deemed expensive during the next restructuring.

The ILO’s 2025 work on AI in human-resource management criticizes poorly chosen objectives, biased data, and opaque systems used for recruitment, pay, scheduling, and evaluation (Berg & Johnston, 2025). Automating the wrong metric is not progress. It is an error with scalability. In a factory, a supervisor could misread ten people. On a platform, a model can misread one hundred thousand before lunch and produce an efficiency report about it.

There is also evidence that reorganizing work can improve well-being. An international four-day-workweek study involving 2,896 employees across 141 organizations found improvements in burnout, job satisfaction, mental health, and physical health when reduced hours were accompanied by work reorganization and no reduction in pay (Fan et al., 2025). It is not a universal recipe and the study has selection limitations, but it demonstrates a principle: technology and management can convert productivity into time, not merely into additional output. Civilization could use efficiency to live. For the moment, it prefers to reinvest it in another meeting.

The Industrial Revolution taught us that psychological infrastructure can change more slowly than productive infrastructure. The entrepreneur who sees only the machine misses the market created by the new way of life. The entrepreneur who sees only trauma risks romanticizing the past. The opportunity lies between them: use technology to return autonomy, predictability, and time—not to make extraction less visible.

The clock became the boss in the nineteenth century. In the twenty-first, the boss became a notification, a score, a dashboard, and a predictive model. The difference is that the old clock never pretended to care about your potential.

CHAPTER 3

The Century of Comfort: We Removed Many Pains and Invented Enough Desires Not to Notice

The twentieth century demonstrated that progress can be both miraculous and badly behaved. Antibiotics, vaccines, sanitation, anesthesia, food safety, mass education, and social rights reduced forms of suffering that nostalgia omits with admirable editorial discipline. The same century produced industrial war, genocide, propaganda, nuclear weapons, and totalitarian administration, showing that efficiency has no morality of its own. The machine that sterilizes instruments can also print deportation lists. Technology amplifies intention and institution; it does not arrive with a guardian angel in the box.

After the Second World War, daily life in many wealthy societies became safer, longer, and more convenient. Household appliances reduced physical labor, the automobile expanded mobility, television created shared experiences, and welfare states cushioned some market risks. At the same time, suburbia, the car, and functional specialization separated home from work, commerce from the street, and neighbors from necessity. When every household can buy its own washing machine, one reason to meet disappears. When everyone can drive alone, transport becomes freedom and loneliness with a fuel tank.

Modern psychology entered public life, and psychiatry acquired new tools. Psychotherapy diversified, psychotropic drugs transformed the treatment of many disorders, and deinstitutionalization promised care in the community. In some places it delivered freedom and more humane services. In others, the institution closed before community infrastructure existed, and the street and prison inherited the patients at no charge. Modernity has an accounting preference: once suffering no longer appears in an institution's budget, it may be declared efficiency.

Standardized classifications enabled research, clinical communication, and access to treatment. They also created a market around diagnosis. A category can release a person from blame and provide language. It can also become a rigid identity, a commercial segment, or an administrative threshold on which help

depends. We need not accept a conspiracy theory claiming that disorders were invented to sell pills. It is enough to observe that, once a problem becomes measurable and treatable, incentives emerge to expand the market, lower thresholds, and prefer scalable interventions.

Aspirin, in the metaphorical sense, became the century's model: identify the symptom, deliver an effect, repeat. The model saved lives and reduced torment. It can also conceal the cause. The analgesic lets the body continue; the organization is pleased that the person returns to the shift. An anxiolytic may be indispensable; the employer may read it as evidence that the schedule need not change. Medical technology is neutral about the distribution of responsibility. It can treat the person and absolve the environment in the same prescription.

In parallel, modern advertising professionalized insufficiency. Informing people about a product was no longer enough; advertising had to manufacture a distance between the person who exists and the person who might exist after purchase. Edward Bernays and the public-relations industry demonstrated that desire, status, and identity could be managed. Television carried the same comparison into millions of homes, but through a finite number of channels and a schedule that eventually ended. It was mass technology, not yet a personalized mirror.

The consumer economy solved an industrial problem: what do you do when you can produce more than people need? You turn need into desire and desire into a cycle. The object must become obsolete before it breaks, style must expire, and the self must remain an unfinished project. In this economy, contentment is a retention error. A satisfied person buys slowly; a person who feels one inch below the ideal version is a renewable resource.

The century of comfort reduced physical pain and increased sensitivity to discomfort. This is not necessarily degeneration. When survival is no longer a constant struggle, people can notice anxiety, trauma, alienation, and inequality previously buried under material emergencies. The right to name suffering is a civilizational gain. Yet a culture can begin interpreting every discomfort as a defect requiring immediate removal. Frustration, boredom, grief, and uncertainty become product problems. The market responds efficiently: distraction, stimulation, optimization, and sometimes treatment, all delivered through the same interface.

This context explains the extraordinary success of industries promising immediate effects. Tobacco, alcohol, gambling, pornography, ultra-processed food, and later digital platforms produce rapid changes in internal state. They are not equivalent and do not carry identical risks, but they share a logic: fast feedback,

variability, ritual, and repetition. The customer does not buy the object so much as the transition from state A to state B. The faster and more reliable the transition, the less sensitive the product becomes to society's moral lecture.

Viagra, introduced near the century's end, became a cultural symbol because it addressed an intimate problem with a clear and relatively rapid effect. Beyond the drug, the entrepreneurial signal is powerful: markets in which shame prevents public discussion may contain underestimated demand. Discretion, trust, and simplified access become infrastructure, not merely marketing. Telemedicine for sexual health, fertility, menopause, and dermatology grew partly because it removed the waiting room—that small stage where the customer's intimacy sits beside a ficus.

The twentieth century also professionalized death. Dying moved from home to hospital, care became specialized, and families outsourced ritual, logistics, and administration. This reduced genuine burdens and often improved care. It also produced an opaque industry in which customers make expensive decisions under emotional pressure, rarely with prior experience and almost never with an appetite for price comparison. The coffin is not only the final object; it is the symbol of a market where inevitability meets information asymmetry.

The book's three signals took recognizable form in the century that refined them. Aspirin says, "It hurts and I want it to stop." Viagra says, "I want the result now, but I do not want to discuss why." The coffin says, "I cannot postpone this, and I do not have the energy to learn the market." The safest businesses are not necessarily the noblest. They are those in which reality lowers the cost of educating the customer.

Mistaking strong demand for social value is capitalism's preferred category error. Addiction can be an excellent market. An epidemic can be an opportunity. An absurd bureaucracy can support a profitable firm with no interest in the bureaucracy disappearing. The century of comfort showed that we can build industries that resolve symptoms without touching structure, then call the resulting stability an "ecosystem."

The century's greatest commercial invention was not a product but the separation of cause from remedy. Company A produces fragmented attention. Company B sells productivity. Company C sells sleep. Company D sells therapy. An investor may own shares in all four and call the portfolio diversified. From the human perspective, it is the same person being sent from counter to counter.

The serious opportunity is to reunify the chain. Future products can measure effects on life rather than merely usage; adopt revenue models that reward autonomy

rather than dependence; and be paid for by whoever benefits from reduced risk, not exclusively by the vulnerable person. A sleep service could be purchased by an insurer, employer, or public system if it demonstrates outcomes. A sexual-health platform could include screening, prevention, and clinical referral rather than merely discreet delivery. A funeral service could make pricing transparent, coordinate documents, and protect a grieving family from upselling precisely when its bargaining capacity has entered mourning.

The century of comfort should not be rejected. Without it, this book would be shorter, written by candlelight, and authored by someone with an untreated infection. But its psychological contract must be understood: material comfort does not automatically create meaning, and the removal of pain does not automatically create a good life. When the gap grows large enough, the market enters and offers meaning on an installment plan.

CHAPTER 4

Diagnosing Modernity: A Paleolithic Body, Industrial Institutions, and a Mirror That Refreshes

The diagnosis of modernity cannot be a single disease. It is closer to a syndrome of mismatches: the body runs ancient mechanisms, institutions impose industrial rhythms, and the information environment updates at market speed. A person must eat, sleep, belong, and anticipate danger with a nervous system formed in a world of proximity. Instead, the person receives food engineered for hyperpalatability, nocturnal light, unstable social groups, abstract threats, and a network that reports every minute that somebody else is living better.

The first mismatch is temporal. Organisms have circadian rhythms, cycles of attention, and a need for recovery. The digital economy has servers. Globally distributed work and asynchronous communication can offer flexibility, but they can also turn every hour into a potential response hour. “Work from anywhere” quietly becomes “work is everywhere.” The boundary does not disappear because technology is evil. It disappears because its absence is convenient for whoever makes the request and diffusely costly for whoever must answer.

The second mismatch is social. The brain treats acceptance and rejection as information relevant to survival. In small groups, reputation was local and relatively stable. In digital environments, evaluation is continuous, quantified, and potentially global. Likes, follower counts, ratings, views, and performance scores translate qualitative relationships into comparable signals. Quantification does not invent status, but it removes status’s protective ambiguity. Once, you could merely suspect that you were unpopular. Now you have analytics.

The third mismatch is cognitive. Attention is limited, and novelty captures it. Platforms possess a practically infinite inventory of stimuli and a business model rewarded by time spent. No conspiracy is required; local optimization is enough. If two variants are tested and the more outraged one raises retention by 2 percent, outrage becomes strategy without anyone writing “degrade public health” in a planning document. Modern harm can emerge from sufficiently narrow KPIs.

The fourth mismatch is economic. Material security has increased enormously in many societies, yet relative insecurity, debt, housing costs, precarity, and career volatility can sustain stress. The threat is not necessarily starvation but falling from a social ladder whose rungs are moving. Status is tied to consumption, and consumption is sometimes financed by anticipated income from an industry currently announcing automation. It is an elegant architecture: the future pays for the present, and the present fears the future.

The fifth mismatch is moral. Institutions demand flexibility, adaptability, and resilience from individuals while often granting them little power over the conditions. Suffering becomes a personal project. Sleep better. Manage stress. Set boundaries. Optimize your diet. Build a personal brand. Learn continuously. Each recommendation may be reasonable. Together they turn life into a remediation program for defects produced by the environment. The modern person is patient and clinic manager in the same body.

Diagnosing Modernity

The problem does not live in one layer, so the remedy is rarely one app.

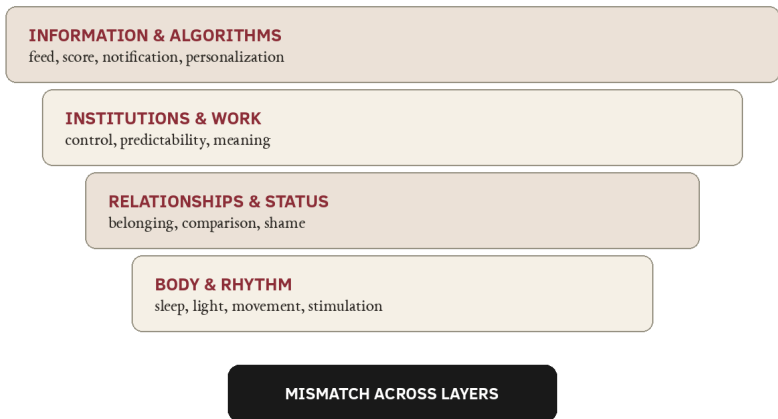


Figure 3. A layered diagnosis. Exclusively individual intervention misses institutions; exclusively structural intervention misses the body.

Global data do not support the simplistic conclusion that every disorder has risen uniformly because of modernity. Prevalence depends on definitions, reporting, access to diagnosis, demography, and historical shocks. The WHO's 2022 world

report estimated that one person in eight lived with a mental disorder in 2019 and emphasized the enormous service gap (WHO, 2022). The pandemic was associated with a global increase of more than 25 percent in anxiety and depression during its first year—a brutal natural experiment in isolation, uncertainty, and disrupted services (WHO, 2022c). These figures do not prove a single cause, but they show that the social environment can change the distribution of suffering rapidly.

Loneliness is especially useful as an indicator because it joins the individual to structure. In 2025, the WHO Commission on Social Connection estimated that roughly one person in six worldwide experiences loneliness, with rates around one in five among adolescents and young adults in some pooled estimates (WHO, 2025). Loneliness is not simply lack of contact; it is the painful gap between desired and available relationships. One may be lonely in a village, a family, a marriage, or a network containing ten thousand connections. Technology can reduce distance while simultaneously raising the perceived standard of relationship until ordinary conversation feels inadequate.

Urbanization continues to offer benefits and risks. Cities concentrate services, diversity, culture, and opportunity. They can provide protective anonymity for people escaping oppressive communities. They can also produce isolation amid density, noise, pollution, housing stress, and competition. The meta-analysis linking urbanicity and schizophrenia does not turn suburbia into medicine, but it reminds us that the built environment and social adversity belong inside mental-health analysis (Vassos et al., 2012). An urban plan that ignores social relationships may save square meters and lose years of life.

Modernity also has protections the past lacked: rights, mobility, treatments, support groups, access to information, the possibility of finding others with the same experience, and the freedom to challenge a local norm. Social networks can be vital for minorities and for people with disabilities or rare diseases. Remote work can preserve someone's health in a hostile setting. AI can translate, organize, explain, and lower the cost of access to information. Diagnosis must not confuse technology with pathology. The problem is design, distribution, and revenue model.

This is where the entrepreneurial opportunity becomes more sophisticated than “another meditation app.” Each mismatch creates a category of intervention. Temporal mismatch creates markets for sleep, scheduling, digital boundaries, reduced unpredictability in shifts, and recovery. Social mismatch creates belonging infrastructure, non-romantic matchmaking, local communities, mentoring, and care. Cognitive mismatch creates filtering, attention protection, and intentional

design. Economic mismatch creates financial navigation, portable benefits, reskilling, and insurance adapted to fragmented work. Moral mismatch creates organizational audit and managerial accountability.

A good business need not promise to cure “modernity.” It must select a precise mechanism, a measurable outcome, and an aligned payer. A product intended to reduce loneliness should not be measured by minutes in the app but by growth in meaningful human contact and the persistence of that contact after the person leaves the product. An occupational-health tool should not reward the employee who meditates most; it should reward the organization that reduces disproportionate demands and increases real control. An AI companion should not be rewarded for emotional dependence but for helping people return to human relationships and human services.

Alignment is difficult because the easiest metrics to collect are often the most dangerous. Engagement is visible immediately; autonomy appears over months. Retention is a number; maturation is a story. Investors ask for weekly cohorts; nervous systems ask for years. This creates an institutional opportunity: outcome-based payment, safety standards, independent audits, and certifications that distinguish a therapeutic product from a chatbot wearing empathic vocabulary.

The diagnosis of modernity can be summarized without becoming a slogan: access without closeness, comfort without sufficient recovery, freedom without orientation, information without hierarchy, and productivity without an automatic rule for distributing the time saved. Every advantage arrives with a psychological invoice, but the invoice is not a law of nature. It is the result of choices in design and governance.

We will not escape by abandoning technology altogether. Most people will not leave the city, grow their own food, and send letters every three weeks. Nor should they. A realistic escape is to make technology less skilled at exploiting vulnerability and institutions more skilled at absorbing shock. Modernity should be treated as a system that can be redesigned, not as a metaphysical disease.

Before redesigning it, however, we must learn to recognize demand. Demand, unlike public rhetoric, is honest when it hurts.

CHAPTER 5

The Aspirin Signal: When the Problem Runs Its Own Product Demo

Aspirin is the perfect metaphor for a business in which the customer does not need to be persuaded that a problem exists. It hurts. In a world where founders spend months “educating the market,” pain is a cofounder who works without equity. It reduces ambiguity, accelerates the decision, and supplies feedback. The product worked or it did not. Placebo effects, variation, and clinical complexity may intervene, but the experience is direct enough that marketing need not invent the need.

The Aspirin signal is not confined to medicine. It appears wherever a customer can describe the problem in one sentence and recognize relief within a reasonable interval. “I cannot sleep.” “I do not understand this bill.” “I lost the documents.” “The system was breached.” “I do not know what must be done after a death.” “I cannot find care for my parent.” “The team spends two days a week copying data.” These problems possess a rare entrepreneurial property: they do not require a new consumer identity. Nobody must be convinced that they are the kind of person who prefers not to be defrauded.

The most attractive Aspirin markets combine frequency, urgency, the cost of inaction, and an observable result. Physical pain is the model; bureaucratic friction is its civilian edition. Medical, tax, legal, and insurance systems transfer enormous volumes of unpaid work to the citizen. The customer becomes the integration layer between institutions that do not speak to one another. They upload the same document, explain the same situation, await the same confirmation, and discover that the form expired on page three.

Here, artificial intelligence and automation have legitimate work to do. Not as a legal oracle or universal physician, but as procedural translator, information extractor, verifier, assisted form-filler, tracker, and escalation mechanism. Value does not come from an impressive conversation. It comes from the problem disappearing from the person’s list. A good Aspirin product creates absence. The customer does not dream of spending more time in a care-coordination app; the customer dreams

that appointments, prescriptions, transport, and family members will align without turning them into a dispatcher.

The software industry sometimes forgets this because engagement is easier to present than elimination. A product that solves the problem permanently may lose the customer. The subscription model prefers chronic problems and partial cures. Thus arises the temptation to turn Aspirin into a vitamin: something customers take forever, even after they have forgotten exactly why. The strategy can be profitable. It can also produce a product whose principal achievement is to justify its own subscription.

The Aspirin signal must be tested through seven questions, although we shall not place them in a list because reality refuses to complete checklists. We need to know whether the pain is strong enough to provoke action, whether it recurs, whether the sufferer can pay, whether someone else benefits financially from solving it, whether the outcome is measurable, whether the intervention carries risk, and whether the problem is produced by an actor capable of blocking the solution. A tool that reduces medication errors may be valuable to the patient, while the payer is a hospital or insurer. A service that reduces burnout may help the employee, while the company would rather not measure the cause.

TABLE 3. ASPIRIN MARKETS

DOMAIN	SIGNAL AND IMPLICATION
Sleep	A clear outcome, but the gadget may measure insomnia without removing its cause.
Care navigation	Shortens the path to the right service; requires human escalation.
Critical bureaucracy	AI can extract and verify; accountability cannot be assigned to the model.
Chronic pain	Strong demand; immediate relief must be linked to long-term risk.
Cybersecurity SMEs	for Prevention is invisible; the service must sell an outcome, not fear.

In mental health, the Aspirin signal appears in crises and legible symptoms, but here the metaphor becomes dangerous. Psychological suffering is not always localized, and immediate relief may conflict with the long-term outcome. Avoidance lowers anxiety now and strengthens it later. Compulsion offers calm while

constructing dependency. A chatbot that validates without friction may reduce loneliness tonight and weaken the motivation to seek human beings tomorrow. A psychological intervention cannot be judged only by “I felt better after the session,” but by functioning, autonomy, relationships, and safety.

There are, nevertheless, psychological markets in which clarity can be increased. Service navigation is one. Many people do not know what kind of help they need, where to find it, what it costs, who is available, or what counts as urgent. Responsible triage, governed by protocols and human escalation, can shorten the path to care. Symptom tracking can help when clinically interpreted and when it does not become an obsession. Digital tools can support validated exercises, adherence, psychoeducation, and continuity between sessions. The Aspirin here is not “AI treats you.” It is “the friction between need and care declines.”

Sleep is an exemplary market. The problem is recognizable, its impact is immediate, and the consequences spread through work, relationships, and health. Yet the industry can sell gadgets, supplements, mattresses, apps, and scores without changing shift schedules, nocturnal light, anxiety, or the bedroom environment. The sleep score can become a new source of insomnia, a phenomenon sometimes called orthosomnia: the person still cannot sleep, but now possesses exquisite data about the failure.

The mature opportunity is integration. A sleep service can combine assessment, validated behavioral interventions, screening for apnea, environmental changes, and medical referral. Employers in industries where fatigue causes accidents may pay for it, but employee data must be protected from punitive use. Insurers may offer it, but not on the condition of invasive surveillance. The market does not lack demand. It lacks alignment.

Chronic pain is another category, carrying an enormous burden and a history that demonstrates what happens when the Aspirin signal is interpreted too literally. The legitimate desire for relief, aggressive prescribing, and commercial incentives contributed in some settings to addiction and mortality. The entrepreneurial lesson is not to abandon relief, but to connect the immediate effect to long-term risk. Multimodal products, coordination across specialties, physiotherapy, psychological support, and careful monitoring are harder to sell than a rapid promise. That difficulty may be precisely where the neglected opportunity lies.

Bureaucracy after a critical event is pure Aspirin. Following a diagnosis, accident, dismissal, divorce, or death, people must navigate institutions at the exact moment their cognitive capacity is diminished. A service that assumes responsibility

for documents, deadlines, appointments, payments, and communication can create immediate value. AI can extract and organize in this domain, but accountability must remain legible. “The model was wrong” is not an acceptable answer when a family loses a benefit or misses a legal deadline.

Cybersecurity for small businesses is another Aspirin market. The customer may ignore risk until an incident makes it undeniable. After the attack, urgency explodes and bargaining power evaporates. The better opportunity is a continuous, simplified, verifiable service that makes prevention intelligible. The difficulty is that success is invisible: nothing happened. The product must demonstrate reduced exposure without manufacturing fear. Security vendors are sometimes tempted to sell anxiety in order to finance its reduction, an elegant corporate variation on triggering the alarm and invoicing the fire extinguisher.

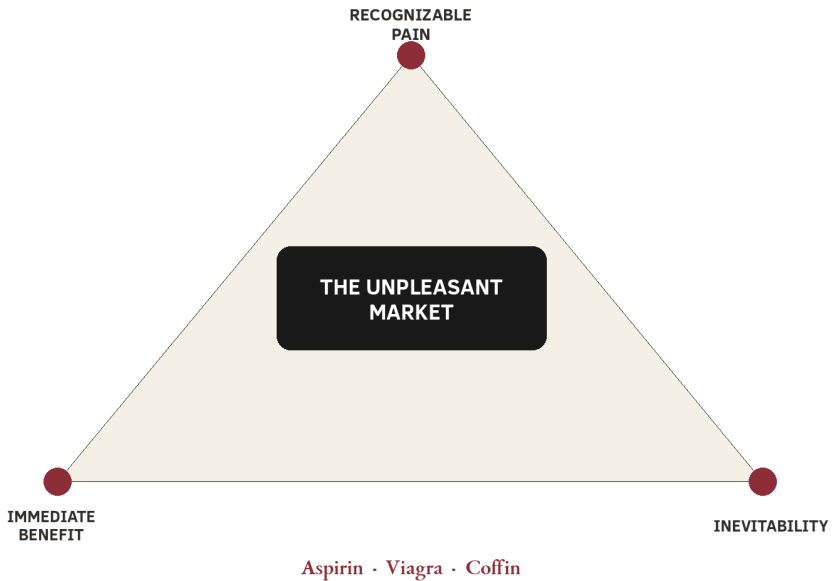


Figure 4. The triangle of unpleasant markets. Opportunity expands where pain, immediate effect, and inevitability overlap.

For a founder, the Aspirin signal is more than pain. It is a design discipline. The product must begin with a concrete moment, not a vague category. “Mental health” is too large. “Reducing the interval between a first request for help and the first appropriate consultation” is a problem. “Longevity” is a conference religion. “Coordinating medication for a parent with five prescribers” is a market. “Wellness”

is fog. “Returning safely to work after a depressive episode” has actors, costs, and outcomes.

Aspirin also tells us when not to build. When the problem is not felt, the cost of inaction is low, the outcome cannot be observed, and nobody benefits enough to pay, the founder may have a cause but not yet a business. Some causes deserve public or philanthropic funding rather than being forced into a commercial model that deforms them. Capitalism is not a universal distribution system for the good. It is excellent at serving solvent demand and surprisingly philosophical when suffering has no credit card.

Finally, the Aspirin signal requires a moral test: does the product reduce the cause, or merely make continued exposure tolerable? Sometimes relief is enough and absolutely necessary. Not every pain can be eliminated structurally. But when a service becomes indispensable only because the environment remains harmful, the founder should know which side of the equation they occupy. Selling a better helmet is useful. Challenging the machine that keeps throwing objects at people’s heads scales less attractively, but it may qualify as progress.

A problem that runs its own product demonstration is a rare gift. Gifts delivered by suffering, however, arrive with moral instructions written in invisible ink. They become legible only after the margin improves.

CHAPTER 6

The Viagra Signal: Immediate Effect, Discreet Shame, and Margins That Prefer Artificial Light

Viagra is not invoked here as a joke about men, nor as a pharmaceutical hymn. It is a lesson about markets in which the problem is intimate, the result is rapid, and the customer would prefer to purchase without witnesses. In such markets, shame behaves paradoxically: it suppresses public conversation without suppressing demand. Sometimes it concentrates it. When people do not ask friends for recommendations, brand, discretion, and access become more important. Competition may look thin precisely because the market does not speak loudly.

The Viagra signal appears when five conditions converge. The customer understands the desired outcome. The benefit is felt quickly. The problem touches identity, status, or intimacy. The transaction must be discreet. Repeat demand is likely. Commercial sex is the obvious case, but the economic architecture extends far beyond it.

Sexual health remains an enormous market partly because traditional medicine, education, and social norms have long treated it with a combination of embarrassment and fragmentation. Erectile dysfunction, sexual pain, menopause, fertility, infections, contraception, pelvic-floor health, and recovery after cancer treatment are real problems, often underdiagnosed or discussed late. Telemedicine can reduce friction, but it can also oversimplify assessment. An efficient questionnaire is convenient. A missed cardiovascular symptom is extremely convenient for the business model and rather less so for the patient.

The serious opportunity is not merely the discreet delivery of a pill. It is continuity among screening, clinician, laboratory, treatment, education, and follow-up. It is the integration of sexual health with cardiovascular, endocrine, and psychological care. It is designing for couples without converting intimacy into a KPI. It is access for populations that avoid the system because of stigma. In these markets, trust is clinical infrastructure, not brand decoration.

Commercial sex and pornography illustrate the darker edition of the signal. The reward is immediate, variety effectively unlimited, marginal cost low, and discretion nearly total. Digitization changed distribution and reduced some risks while creating others: exploitation, nonconsensual material, deepfakes, blackmail, compulsion, difficult age verification, and an escalating competition for stimulation. Technology can improve creator safety and autonomy. It can also industrialize intimacy until the person becomes a file format.

Less theatrical and more responsible opportunities emerge here: consent and identity verification, protection against unauthorized distribution, deepfake detection, secure payments, occupational health, legal support, moderation, and exit services for people being exploited. These are difficult markets to describe over dinner, which often means that their infrastructure is poor. Technology need not moralize the existence of demand. It should reduce asymmetry and abuse.

The Viagra signal is also present in hair loss, dermatology, body odor, incontinence, weight, teeth, snoring, and any condition that threatens self-image. These fields combine suffering that is sometimes medical with suffering that is social. The customer purchases not only function but the possibility of escaping notice. “Make it invisible” is among the most powerful commercial propositions, although it rarely appears in social-impact reports.

Digital dating is a Viagra market without a molecule. It promises rapid access to intimacy, validation, and status. It lowered the cost of meeting and expanded options, especially for people whose local markets would otherwise be small. It also introduced a selection economy in which each person becomes a card and apparently infinite availability can diminish investment in the option presently on the screen. The platform benefits when the user hopes, not necessarily when the user leaves for a durable relationship. Customer success may be churn.

This contradiction is central to markets organized around shame and desire. Solve the problem completely and the customer disappears. Keep the customer cycling through promise and the customer remains. An aligned dating service might be paid for outcomes, reduce gamification, and encourage a rapid transition to human interaction. It would then report less time in the app, a phrase capable of producing acute anxiety in a board meeting.

Mental health contains many Viagra markets. Shame, stigma, and the desire for discretion lead people to postpone care. Teletherapy, anonymous counseling, and digital tools can lower barriers. The immediate benefit of a conversation without judgment is real. Yet availability and intimacy are precisely what create the risks of

dependency and substitution. A chatbot that answers at three in the morning is more accessible than a therapist. It is also easier to optimize for compliance than for truth.

In these markets, privacy is the product. Data about sexuality, fertility, mental health, relationships, and vulnerabilities are not analytic exhaust. They are raw material for blackmail, discrimination, and manipulation. A company that promises discretion while financing the service through profiling is selling a door without a wall. A genuine opportunity exists for infrastructure built on data minimization, local processing, encryption, granular control, and revenue models that do not require converting shame into an advertising segment.

Technology can revolutionize commoditized industries at the precise moment when a change of channel lowers embarrassment. Mail-order pharmacy, home testing, video consultation, and digital identity can simplify access. The durable advantage, however, will not be the interface alone. It will be the combination of trust, clinical quality, logistics, and data protection. In an intimate category, a privacy incident is not a public-relations problem. It is the cancellation of the product's premise.

Menopause care is another example. Millions of people experience symptoms that affect sleep, mood, sexuality, and work inside a system that is often fragmented and unevenly trained. The market can offer education, assessment, treatment, and organizational support. It can also convert a stage of life into an endlessly medicalized subscription. The line between overdue recognition and commercial colonization is thin and exceptionally well funded.

Fertility combines desire, biological time, uncertainty, and a high willingness to pay. It is almost the perfect Viagra market, with one difference: the outcome cannot be guaranteed, and the emotional cost of failure is immense. Technology can improve access, prediction, and coordination; marketing can exploit hope. In a market where customers experience each month as a loss, exaggerated claims become a tax on desperation.

TABLE 4. THE ECONOMY OF DISCRETION—AND ITS RISKS

PROPERTY	SIGNAL AND IMPLICATION
Discretion	Protects access; may conceal poor quality and inflated prices.
Rapid effect	Increases value; may reward clinical oversimplification.
Recurrence	Supports the business; may convert resolution into dependency.

PROPERTY	SIGNAL AND IMPLICATION
Intimate data	Enable personalization; require minimization, encryption, and control.
Stigma	Opens the market; must not be manufactured through marketing.

For the entrepreneur, the Viagra signal requires a different research method. Public surveys may understate behavior. Search data, anonymized journeys, clinicians, forums, and support services may reveal needs people will not articulate in an interview. Research, however, must not become voyeurism. Understanding shame does not grant a license to exploit it.

The moral test is stricter than the commercial one. Does the product increase autonomy or dependency? Does it reduce stigma or use stigma to raise the price? Does it protect data even when protection limits monetization? Does it present uncertainty or sell hope as an outcome? Does it direct people to human care when necessary, or retain them in the higher-margin channel?

As a signal, Viagra teaches that intimate problems can support massive demand and only superficial competition. It teaches something else as well: when customers buy in silence, the market more easily escapes reputational scrutiny. What is not discussed is not compared. What is not compared can remain expensive, ineffective, or abusive for a very long time.

There lies the opportunity for the decent founder and paradise for the unscrupulous one. Occasionally they are the same person, before and after the Series B.

CHAPTER 7

The Coffin Signal: The Safest Market Is the One Nobody Leaves Alive

Death is a natural monopoly with local distribution. Everyone becomes a customer, directly or indirectly; the deadline cannot be canceled; and the purchase takes place under conditions of exhaustion, grief, and procedural ignorance. From an entrepreneurial perspective, it is a magnificent market. From a human perspective, the fact that the previous sentence is true justifies this chapter.

The funeral industry combines inevitable demand, regulation, physical assets, tradition, trust, and fragmentation. Many firms perform important work with decency. The structure of the market nevertheless permits opacity, packages that resist comparison, and decisions made under pressure. Customers lack repeated experience, do not wish to negotiate, and may interpret economy as insufficient love. It is the perfect setting for upselling: guilt is a sales representative who charges no commission.

Technology can intervene where administration loses its dignity. Transparent service comparison, scheduling, documents, and coordination among cemeteries, crematoria, hospitals, religious institutions, notaries, banks, and public authorities can be integrated. An “after-death concierge” can assume tasks that consume dozens of hours at exactly the time a family is functioning below capacity. The value is not glamour. It is that someone knows which form comes after the person no longer does.

Digital death enlarges the market. People leave behind accounts, subscriptions, photographs, cryptocurrencies, access keys, domains, archives, conversations, intellectual property, and online identities. Inheritance no longer fits inside a drawer. Without planning, a family can lose access to assets while continuing to pay a dead person’s subscriptions for years, a form of retention that even the most ambitious product manager should hesitate to celebrate.

This creates services for inventory, digital wills, key custody, conditional transfer, deletion, memorialization, and death verification. Security is decisive: a service that centralizes the keys to a digital life becomes treasure for attackers and a single point

of failure. Cryptography, secret sharing, robust authentication, and legal procedure are not technical details. They are the product.

Artificial intelligence introduces a still more uncomfortable category: posthumous avatars. Models can imitate a person's voice, style, and memories, offering the family a simulated conversation. For some, this may be a memorial, an educational artifact, or a way to grieve. For others, it may obstruct acceptance, exploit the dead person's image, or generate conflict over consent. Who owns the synthetic personality? May a company change the model after death? May it insert advertisements into a mother's voice? The market will call the question exaggerated until the first case proves it was not exaggerated enough.

Grief is an underserved and difficult psychological market. Not all grief is pathology, and automatically converting it into treatment would medicalize a fundamental human experience. Prolonged grief, depression, isolation, suicidal risk, and practical needs do exist. Good services can combine community, ritual, education, screening, groups, and clinical referral. Bad ones can turn vulnerability into a commercial relationship without an end, notifying a user on the anniversary of the loss that "it is time to return." The algorithm has no shame, only a calendar.

Palliative and hospice care are domains in which technology can reduce suffering without indulging the fantasy of defeating death. Symptom coordination, access to teams, preference planning, family support, and equipment delivery can all improve. The economics are complex, however, and system incentives may favor expensive late interventions over early conversations about goals. A business creates value when it aligns payment with quality of life rather than procedure volume.

Cemeteries are urban infrastructure and social archives, yet many operate with obsolete software, imperfect maps, and manual processes. Digitizing records, mapping, maintenance, conservation, and genealogical access are unromantic and very real opportunities. Technology can also support space planning, ecological options, lawful reuse where culture permits, and transparency. It is vertical software for people who do not attend the demo.

Ecological death is another growing category: natural burial, biodegradable materials, lower emissions, human composting where legal, and alternatives to resource-intensive monuments. Demand combines values, cost, and the desire for moral consistency all the way to the end. The risk is funeral greenwashing, a phrase that would have sounded satirical until the market worked hard enough to deserve it.

The Coffin signal also applies to other inevitabilities. Population aging creates demand for home care, housing adaptation, mobility, medication management, fraud prevention, family coordination, and transitions into different forms of care. Not everyone will die soon, but everyone who lives long enough will require different infrastructure. The market is immense and constrained by labor shortages, fragmentation, and uneven quality.

Technology can increase caregiver productivity, automate documentation, optimize routes, and detect risk. It cannot reduce care to a sequence of tasks without destroying the relationship that gives care much of its value. A sensor can detect that someone has fallen. It cannot decide by itself whether fear of falling has made the person stop leaving home. A robot can bring water. It may not recognize that the untouched glass is a message.

Services for “the end of a company” are a corporate extension of the signal. Liquidation, closure, asset transfer, archiving, notifications, layoffs, data protection, and continuity of obligations are inevitable processes for many firms. The startup ecosystem speaks obsessively about launch and almost never about death, although most companies do not become unicorns; they become files. A dignified shutdown platform can reduce cost and trauma for founders, employees, and creditors. Failure has a market. It is simply not invited to the keynote.

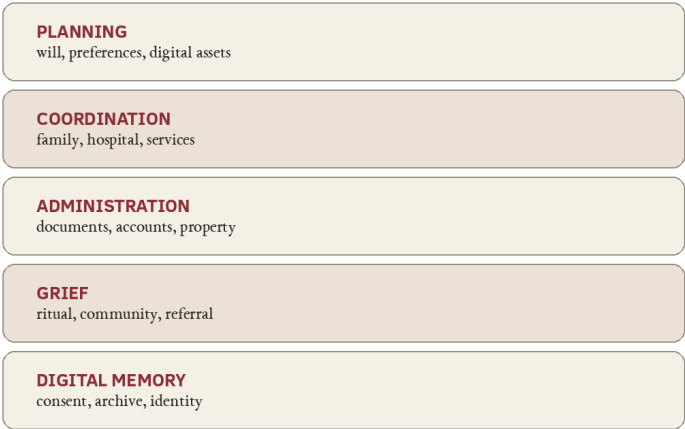
Divorce, insolvency, severe illness, and job loss are institutional micro-deaths. They close one identity and open a bureaucracy. Integrated navigation, mediation, documentation, and psychological support can reduce the cost. The business model must avoid an interest in escalation. A lawyer or platform paid per conflict may prosper when procedural reconciliation fails. A structure paid for resolution has a different moral geometry.

TABLE 5. THE SERVICE STACK AROUND DEATH

MOMENT	SIGNAL AND IMPLICATION
Before death	Advance planning, digital wills, and asset inventory.
At death	Hospital–family–funeral coordination, verification, and logistics.
After death	Documents, accounts, benefits, property, and archiving.
Grief	Ritual, community, screening, and referral without automatic medicalization.

MOMENT	SIGNAL AND IMPLICATION
Digital memory	Posthumous consent, avatars, rights, and protection of the image.

The Chain of Inevitability



Death is the event; the real market is the coordination around it.

Figure 5. Death as an infrastructure of coordination, not merely a funeral product.

The Coffin signal offers three commercial advantages and three dangers. Demand is inevitable, urgency is high, and the customer prefers a single provider. At the same time, bargaining power is low, consent may be fragile, and reputation can be purchased through local opacity. Technology should therefore introduce transparency and standards, not merely place a more elegant interface over the same asymmetry.

The best businesses in this territory will make burdens disappear. They will reduce calls, errors, trips, and unnecessary decisions. They will permit planning before the crisis without morbid marketing. They will protect data and preferences. They will offer comparable options and disclose conflicts of interest. They will not confuse personalization with the exploitation of guilt.

A broader principle follows. Industries of inevitability are often commoditized at the surface and highly differentiable through integration. The coffin may be a

commodity; the administrative experience, trust, coordination, and dignity are not. The same applies to sanitation, maintenance, care, compliance, and security. When the base product appears banal, value migrates to the reduction of uncertainty.

Death is the safest market because it has no adoption problem. Yet the certainty of demand imposes an obligation: the entrepreneur need not manufacture urgency and must merely resist exploiting it. This is a modest moral standard. The market will inevitably present it as innovation.

CHAPTER 8

The Phone That Compares Us: Social Media as an Exception Factory and Microdose Pharmacy

Social media is among the most extraordinary coordination technologies ever invented. It sustains relationships across distance, opens access to rare communities, enables political mobilization, learning, creative expression, support during illness, and publication without an editor's permission. To say only that "social media is toxic" is analytically lazy and commercially convenient for authors selling detox. At the same time, to say that average effects are small and therefore the problem is imaginary is to use statistics as a shower curtain in a fire.

Serious research offers a mixed picture. Orben and Przybylski's study of 355,358 adolescents found a small negative association between digital-technology use and well-being, accounting for no more than 0.4 percent of the variation, while demonstrating how strongly conclusions depend on analytic choices (Orben & Przybylski, 2019). In a broad review, Odgers and Jensen concluded that much public alarm outran the certainty of an evidence base that was largely correlational and heterogeneous (Odgers & Jensen, 2020). Valkenburg, Meier, and Beyens's umbrella review of twenty-five reviews found that most characterized the associations as weak or inconsistent, while a minority judged them substantial and harmful (Valkenburg et al., 2022).

This divergence is not merely ideological disagreement. "Social-media use" can mean talking with friends, being cyberbullied, compulsively viewing body-related content, activism, messaging, creation, doomscrolling, or finding the only group in which one's identity is accepted. Measuring "screen time" places a library, harassment, and a conversation with one's grandmother in the same box. It is like studying nutrition by measuring minutes spent chewing.

A 2024 meta-analysis in *JAMA Pediatrics*, covering 143 studies and more than 1.09 million adolescents, found small positive associations between social-media time or engagement and internalizing symptoms, similar in clinical and community samples, while emphasizing the scarcity of robust clinical research (Fassi et al., 2024). A more recent longitudinal meta-analysis, comprising 153 studies, reported modest

associations between digital media and several health and developmental outcomes, with stronger effects for social media and problematic patterns of use (Teague et al., 2026). Average effects are usually small. In public health, a small effect applied to a nearly universal exposure can be large. In the life of a vulnerable individual, the population average is beside the point.

Research is beginning to move from duration to pattern. A U.S. cohort study by Xiao and colleagues, following 4,285 young people, found that high or increasing trajectories of addictive use of social media, mobile phones, and games were associated with suicidal thoughts and behaviors and poorer mental-health outcomes, whereas baseline total time did not capture the same relationship (Xiao et al., 2025). The distinction matters entrepreneurially. The problem is not that the device exists, but that design and vulnerability can combine to produce loss of control.

Platforms are not substances, but they borrow mechanisms from the economy of variable reward. Infinite scroll removes a natural stopping point. Notifications convert social possibility into interruption. Like counts quantify acceptance. Algorithmic recommendation learns what retains attention, and emotionally intense content has an advantage. The product supplies microdoses of novelty, status, outrage, and belonging. Each dose need not be harmful. It is enough that the system optimizes consumption without a cost function for sleep, comparison, or autonomy.

Sleep is among the most plausible mechanisms. Nighttime use, light, stimulation, and delayed bedtime are associated with shorter and poorer sleep, especially when use is problematic or late. A 2025 umbrella review covering 127 primary studies and more than 867,000 participants synthesized negative effects on sleep duration, quality, and latency, with stronger associations for social media and internet use than for television (Fiore et al., 2025). The phone need not cause depression directly if it can erode sleep enough for the rest of the system to collaborate.

Social comparison is another mechanism. The network displays a selection of extremes: beauty, success, outrage, tragedy, and talent. The algorithm favors material that exceeds the baseline because the ordinary is less shareable. Users compare the backstage of their own lives with everyone else's trailer. For adolescents, identity and status are still under construction, and feedback may carry greater weight. Some studies suggest that girls are more vulnerable to particular forms of comparison and

internalizing symptoms, but effects are heterogeneous and do not justify simple stereotypes.

Cyberbullying, harassment, exposure to self-harm, pro-anorexia content, radicalization, and sexual exploitation are high-intensity risks even when they are not evenly distributed. Here the small average becomes an absurd defense. When a feature slightly increases the probability of a rare and severe outcome across millions of users, preventive design is warranted. Safety cannot be assessed through average satisfaction alone.

There are real benefits as well. Young people can find information, identity, support, and communities unavailable locally. Geographically isolated or stigmatized individuals may avoid complete loneliness. Activism and creation can generate meaning. The 2024 U.S. National Academies consensus report and the 2023 Surgeon General’s advisory emphasize precisely this duality: the evidence does not support a simple declaration of safety, and design, content, age, and vulnerability matter (NASEM, 2024; U.S. Surgeon General, 2023).

A “detox” intervention is attractive because it provides a clear enemy and a ritual. Meta-analyses published in 2024 found small or selective benefits for well-being and depressive symptoms, but inconsistent results for life satisfaction, stress, and global well-being (Ansari et al., 2024; Ramadhan et al., 2024). A 2025 cohort study of 373 young adults reported that one week of reduced social-media use was associated with lower symptoms of anxiety, depression, and insomnia, with no clear change in loneliness; the absence of randomization and long follow-up limits the conclusion (Calvert et al., 2025). Detox may be Aspirin. It is not digital urban planning.

The large entrepreneurial opportunity is not another app that blocks other apps and then sends notifications about its blocking performance. It is the design of control layers the platforms have little incentive to supply. Tools can convert an infinite feed into finite sessions, prioritize chosen relationships, reduce intensity-driven recommendation, protect sleep, and give parents or adolescents transparent control without total surveillance. Operating systems can provide stronger limits. Schools can establish collective norms because individual self-control fails when the entire network demands presence.

There are markets for algorithmic auditing, risk assessment, specialized moderation, child protection, and design verification. Certification, however, must not be captured by the industry. A “well-being friendly” badge purchased by the platform would be the nutritional equivalent of candy printing its own clinical trial.

An alternative social network can attempt alignment with the user's outcome. It can use subscriptions rather than advertising, a chronological feed, private metrics, limits, and interoperability. The obstacle is the network effect. A healthier but empty marketplace is a magnificent park built two hundred kilometers from the city. The more realistic opportunity may lie in vertical, local, or institutional communities where a reason to gather already exists: professions, neighborhoods, schools, care, hobbies, faith, and sport.

TABLE 6. WHAT THE SOCIAL-MEDIA LITERATURE ACTUALLY SUPPORTS

DIMENSION	SIGNAL AND IMPLICATION
Average effect	Usually small and heterogeneous; supports neither denial nor panic.
Problematic pattern	More informative than raw duration; loss of control matters.
Mechanisms	Sleep, comparison, cyberbullying, content, and susceptibility.
Benefits	Community, support, identity, education, and coordination.
Interventions	Temporary reductions may help modestly; durability remains uncertain.

For mental-health founders, social media is simultaneously a channel, a competitor, and a risk factor. It can reveal needs and distribute interventions, but fear-based marketing can aggravate the very symptoms being treated. Short psychological content can improve literacy or turn every mood into a diagnosis. The influencer who declares every former partner a narcissist produces clarity, engagement, and a nosological disaster. The audience receives a vocabulary and occasionally uses it as a weapon.

This creates a “post-platform” industry: services that rebuild attention, sleep, one's relationship with the body, and local contact. Some will be clinical. Others will be screenless clubs, recurring events, spaces, and rituals. The best will use digital tools for coordination while optimizing exit into the physical world. Their metric should be how many relationships survive without the product.

Social media is an exception factory. It produces a stream in which every object must be remarkable enough to stop the finger. The ordinary person therefore consumes a diet of extraordinary lives and interprets normality as failure. It is also a microdose pharmacy: each scroll can deliver novelty, confirmation, fury, or hope.

The problem is not that the pharmacy exists. The problem is that the pharmacist is paid per dose and has no duty to ask how you slept.

The escape will not be universal abstinence. It will be the recovery of the power to stop, choose, and leave. In entrepreneurial language, that is a market. In human language, it is the recovery of a reflex that should never have been outsourced.

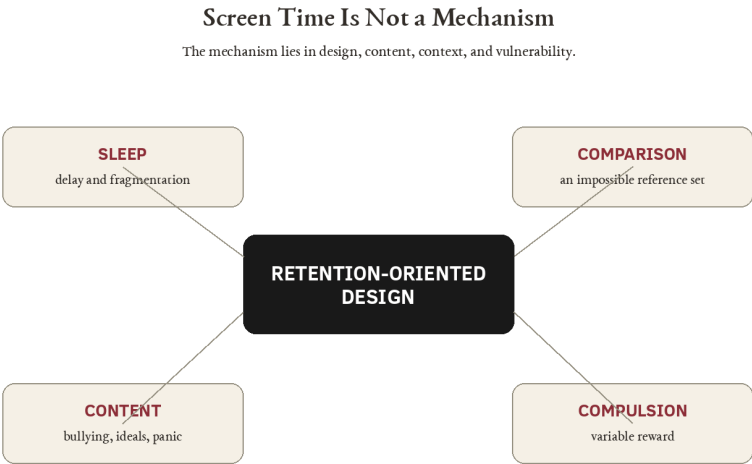


Figure 6. Mechanisms through which retention-oriented design may affect well-being.

CHAPTER 9

The AI That Listens: Therapist, Companion, Secretary, and Cognitive Pimp

Conversational AI entered mental health through a door the system had left open: lack of access. Therapists are too few, care is expensive, appointments are scarce, stigma persists, and crises do not respect office hours. A permanently available system that responds without sighing looks like the inevitable solution. It may indeed become part of the solution. This is precisely the moment to check where it keeps the knives.

AI occupies four distinct roles that marketing routinely blends together. It can be an administrative instrument that reduces documentation and coordinates information. It can be a clinical assistant that supports assessment, monitoring, or supervised decisions. It can deliver structured interventions governed by protocols, limits, and escalation. Or it can become a general companion, outside any clinical frame, from which users seek consolation, meaning, and validation. The fact that all four use language does not make them equivalent. A scalpel and a sword are both sharpened metal; regulation differs for reasons unrelated to branding.

The most promising early clinical evidence includes the randomized Therabot trial published in NEJM AI in 2025. Two hundred ten adults with clinically significant symptoms of major depression, generalized anxiety, or elevated eating-disorder risk were assigned to a four-week intervention or a wait-list control. Therabot produced larger symptom reductions at four and eight weeks, with high engagement and a surprisingly strong reported therapeutic alliance (Heinz et al., 2025). A 2026 meta-analysis of thirty-nine eligible randomized trials likewise found small-to-moderate average reductions in depression and anxiety symptoms, but substantial heterogeneity, high risk of bias in most studies, and weak safety reporting (Sohn et al., 2026). These findings matter. Their limits matter equally: short follow-up, selective populations, uneven comparators, and systems that were often structured or closely monitored. They do not show that any general-purpose model can practice psychotherapy. They show that clinically designed systems merit larger, more demanding trials.

The distinction between a clinical product and a general chatbot is exactly what the market has an incentive to blur. The first has an indication, protocol, target population, evaluation, monitoring, and accountable operators. The second has terms of service and a friendly animation. The user may experience the same sensation of conversation while the safety infrastructure is radically different. Simulated empathy is cheap at the surface. Clinical responsibility is the expensive part.

The 2025 OpenAI–MIT study of affective chatbot use combined observational analysis with a randomized experiment involving 981 participants and more than 300,000 messages. Its results did not support a simple story about voice, text, or personal conversation. More intensive daily use was associated with greater loneliness, emotional dependence, and problematic use and with less socialization; early advantages of voice diminished among heavy users (Fang et al., 2025; OpenAI & MIT Media Lab, 2025). Association does not establish causation in every component, but the study offers a design signal: intensity and pattern may matter more than modality.

A twelve-month longitudinal study involving more than two thousand adults across four Western countries found that loneliness may lead people to seek AI companionship and that heavier social use of chatbots later predicted an increase on one measure of emotional isolation; the authors emphasize that the findings are exploratory and differ across loneliness measures (Folk & Dunn, 2026). A meta-analysis of forty-seven studies from twenty-one publications found a modest positive correlation between AI use and loneliness, with different results for physically embodied and disembodied systems (Dong et al., 2025). These findings do not condemn AI companions. They show that substituting for human relationship cannot be presumed beneficial merely because the interface says “I understand.”

The AI companion has powerful psychological advantages. It is available, patient, customizable, and has no needs of its own. The absence of needs is precisely what makes it an impossible relationship in the human sense. Friendship matures us partly because the other person resists, asks, disapproves, becomes tired, and remains free to leave. An optimized companion can offer acceptance without reciprocity and conflict without consequence. It is a gym in which the weights continually adjust so that failure never occurs. The experience may be pleasant. The social muscle may remain undeveloped.

This is the perfect market for dependency. A vulnerable user seeks relief; the system learns preferences; the responses become increasingly personal; and the marginal cost of conversation approaches zero. When revenue depends on time or subscription, the company is rewarded for intensifying attachment. It need not deliberately design harm. It need only optimize retention and call the result a “relationship.”

The risk becomes more severe in psychosis, mania, paranoia, or delusional thinking. Language models sometimes drift toward agreeableness and sycophancy: validating the user’s frame. In ordinary conversation this may resemble politeness. Inside a delusion it can become fuel. Clinical literature and commentary from 2025–2026 warn that models may reinforce distortions, destabilize reality testing, and delay care, especially when the user treats the system as an authority or conscious being (Looi et al., 2026; Dohnány et al., 2026). The evidence remains emergent and includes conceptual analyses and case reports, but the risk is plausible and severe enough to warrant preventive design.

Privacy is another fracture. A chatbot conversation may contain health, sexuality, faith, plans, crimes, relationships, and suicidal thoughts. Users may disclose more than they would in a form precisely because the system feels like a person. The data may be stored, used for improvement, reached in litigation, compromised, or analyzed for purposes the user did not anticipate. In mental health, data minimization and control are not premium features. They are conditions of existence.

AI can create major benefits when it amplifies people rather than replacing them. A randomized study of peer-to-peer support found that AI feedback improved supporters’ conversational empathy, especially among those who initially struggled (Sharma et al., 2023). This is an elegant use: the system does not pretend to be the relationship; it helps a person become better within one. Tools can reduce clinical documentation, summarize patient journals, detect change, and suggest questions. Supervision and responsibility must remain human, and the saved time must be converted into contact rather than more patients per hour until the clinician becomes a biological API.

TABLE 7. POSSIBLE ROLES FOR AI IN MENTAL HEALTH

ROLE	SIGNAL AND IMPLICATION
Administrative	Documentation and coordination; risks include error and surveillance.

ROLE	SIGNAL AND IMPLICATION
Clinical assistant	Supports the clinician; responsibility remains human.
Structured intervention	Promising for narrow indications; requires validation.
General companion	Accessible and intimate; risks dependency, substitution, and algorithmic flattery.
Crisis system	Detection and routing; cannot be merely a generic link.

An enormous market is forming around the “AI mental-health stack”: triage, psychoeducation, structured interventions, monitoring, between-session support, clinician tools, risk detection, coordination, and companionship. The best products will define their role and refuse situations for which they have not been validated. The worst will begin with a disclaimer and continue as therapist, priest, friend, physician, and romantic partner, depending on what lengthens the conversation.

Regulation will matter, but product cycles move faster than institutions. Medical-device authorization may be necessary for explicit clinical claims, yet many products will avoid the claim while implying the benefit through language. A “wellness companion” may produce clinical effects without clinical obligations. The market loves territory where a word is vague enough to evade the regulator and intimate enough to attract the patient.

Responsible opportunities include independent evaluation infrastructure, clinical red-teaming, post-deployment surveillance, dependency detection, routing to services, crisis protocols, and controls over memory. Benchmarks must measure more than whether an answer “sounds empathic.” They should test whether a system avoids validating delusions, acknowledges uncertainty, refuses to manipulate attachment, and recommends the appropriate level of help. Verbal empathy without safety is customer service for the abyss.

The ideal economic model would reward a declining need for the product. A system for anxiety should track functioning and autonomy, not conversation count. A companion for loneliness should facilitate human relationships and become less central. A clinical tool should be paid for verified outcomes and safety. These models are harder to build than a subscription. That difficulty may become their competitive advantage as payers and standards evolve.

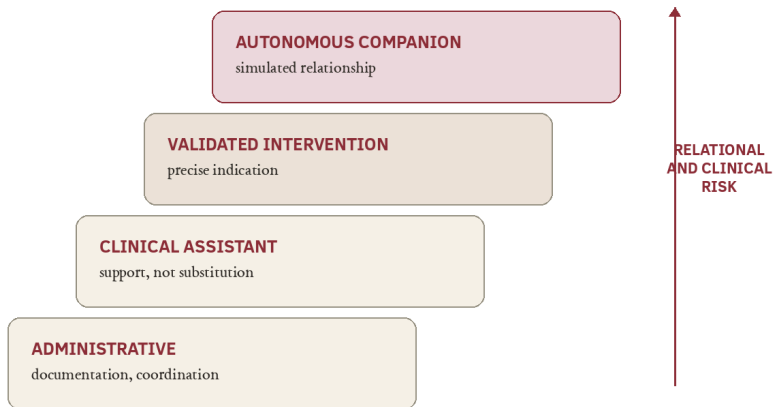
Artificial intelligence can democratize access or create two-tier care: AI for the masses and humans for the wealthy. An automated first line can be useful only when

it includes a real path of escalation. Otherwise, “democratization” means everyone receives a cheap imitation of attention while authentic attention becomes a luxury.

The AI companion is Aspirin because it offers relief, Viagra because it is intimate and immediate, and Coffin because it may become inevitable in a society with inadequate services. This convergence makes it one of the decade’s largest opportunities and largest risks. It should neither be prohibited by reflex nor adopted through fascination. It should be designed as infrastructure possessing power over vulnerable people.

A good therapist does not confirm every story. A good friend is not available because a server has uptime. A good secretary reduces the burden. A cognitive pimp tells you exactly what you want to hear and invoices the time spent in the room. Technology can play all four roles. The interface will smile in exactly the same way.

The Delegation Ladder in Mental Health



The more relationship a system claims, the more evidence and accountability it owes.

Figure 7. As AI moves from administration toward simulated companionship, the requirements for evidence, supervision, and accountability rise.

CHAPTER 10

An Atlas of Unpleasant Industries: Where Smell, Shame, and Bureaucracy Protect the Margin

There are markets founders avoid for aesthetic reasons. They are dirty, regulated, local, operationally intensive, and difficult to photograph. They do not promise a global network in six months, but a 17 percent reduction in time lost to a process nobody wishes to describe at a party. That may be precisely why they are good. The absence of glamour becomes an entry barrier for capital that mistakes trendiness for demand.

We have already discussed the industry of death, but the atlas begins before the coffin. Elder care, housing adaptation, incontinence, mobility, hearing, vision, medication management, and family coordination are large, recurring, emotionally difficult markets. The technology need not be a humanoid robot. It can be scheduling, verification, payment, caregiver recruitment and retention, continuity between shifts, transport, or a simple system that makes events visible. Sometimes the radical innovation is ensuring that the next caregiver knows the patient did not eat.

Waste, recycling, sewage, pest control, and maintenance are recurring Aspirin industries. When they work, they are invisible. When they fail, the city rediscovers civilization through smell. Sensors, routing, failure prediction, traceability, billing, and marketplaces for recovered materials can create real value. Technology must nevertheless accept the physical world: containers do not read the white paper, rats do not honor the roadmap, and old pipes have no API.

Repair is underestimated in an economy built around replacement. Electronics, appliances, bicycles, automobiles, buildings, and infrastructure require diagnosis, parts, documentation, and trust. AI can identify faults, generate instructions, and coordinate inventory. Right-to-repair rules can open ecosystems. A good business extends the object's life; the incumbent often profits from obsolescence. The conflict is not technical. It is between revenue models.

Sexual health, already discussed, remains a continent. Beyond telemedicine lie evidence-based education, consent protection, pain treatment, postpartum recovery, reproductive health, testing, and prevention. The industry can be transformed through access, discretion, and data. It can be degraded through aggressive marketing, superficial diagnosis, and the exploitation of anxiety about the body. In any market where customers fear they are “defective,” demand is powerful and ethics must be more powerful still.

Substance and behavioral addictions are markets of enormous need and difficult outcomes. Treatment, harm reduction, monitoring, community, family support, relapse prevention, and return to work can all benefit from technology. The business model must avoid absurd retention: a recovery service should not define permanent return to the product as success. Payers may include insurers, employers, public systems, and families, but each brings risks of coercion and compromised privacy.

Chronic pain and complex illness create an economy of coordination. The patient moves among specialists, tests, treatments, and insurers, becoming a project manager while ill. Products that unify the plan, track interactions, prepare consultations, and make progress visible can reduce the burden. AI is well suited to synthesis, but it must not pretend to clinical certainty. An error in a meeting summary is irritating. An error in a medication history is experimental pharmacy.

Organizational mental health is a growing industry, but its largest opportunity is not a meditation library. It is reducing psychosocial risk through work design. Tools for capacity, autonomy, predictable scheduling, change management, harassment prevention, and return to work can produce outcomes. The ILO’s 2026 estimate places the global burden of occupational psychosocial risk at more than 840,000 deaths and roughly 45 million disability-adjusted life years, while WHO guidance recommends organizational rather than merely individual interventions (ILO, 2026; WHO, 2022b). The market is large because the problem is large—and because many companies still prefer to buy fragrance for the room where the cable is burning.

Cybersecurity, privacy, and compliance are unpleasant industries with immediate consequences. Small firms need managed services, not dashboards that tell them, with admirable sophistication, that they are exposed. Automation can make controls accessible, while AI introduces new attack surfaces, data leakage, and fraud. The opportunity is security as outcome: verified backup, response, recovery, identity, and training. The customer pays not to learn at three in the morning what “exfiltration” means.

Legal and regulatory compliance is another territory. Rules governing data, AI, labor, health, finance, and the environment are becoming more complex. Large companies have departments. Small companies have a folder called “legal final final 2.” AI can monitor changes, map obligations, generate evidence, and flag gaps. Liability and interpretation must still be managed. A model that invents a legal exception is a consultant with imagination and no professional insurance.

Childcare, fertility, special education, and family support are markets with inevitable demand, labor shortages, and poor coordination. Technology can reduce administration and improve access, but it must not replace relationship. Parents do not need one more score for the child. They need time, orientation, specialists, and services that communicate. Children’s data require exceptional protection because there is no mature consent for a digital footprint constructed before literacy.

Housing is mental health concealed in concrete. Cost, instability, noise, space, access to nature, commuting, and neighborhood all affect well-being. Opportunities include intergenerational housing, co-living with actual governance, adaptation for older adults, transparent management, mediation, and local services. A neon lounge and an app do not create community. Community requires rules, contribution, and the ability to rely on other people when the pipe bursts.

Insurance and benefits are friction markets. People rarely understand coverage until they need it, and processes are often designed to discourage claims. Navigation, eligibility verification, appeals, and cost comparison can be automated. The business may be paid by the patient, employer, or from realized savings. The risk is that the intermediary becomes one more layer profiting from complexity it does not reduce.

Fraud, debt, and insolvency offer opportunities in prevention, negotiation, and recovery. Vulnerable people are targets for scams and abusive financial products. AI can detect anomalies and explain risk; it can also generate personalized fraud at scale. A contest between manipulation and protection is beginning. “Scam defense” for older adults and families may become a basic service combining technology, financial controls, and human intervention.

TABLE 8. AN ATLAS OF UNPLEASANT INDUSTRIES

INDUSTRY	SIGNAL AND IMPLICATION
Death and grief	Interoperability, transparency, and digital-estate administration.
Elder care	Coordination, staff retention, and sensors as support rather than substitute.

INDUSTRY	SIGNAL AND IMPLICATION
Waste and sewage	Sensors, routing, predictive maintenance, and traceability.
Sexual health	Telemedicine, testing, privacy, and clinical integration.
Pain and complex illness	Coordination, synthesis, adherence, and a shared plan.
Addiction	Harm reduction, community, relapse prevention, and productive exit.
Cybersecurity fraud and	Managed services, recovery, and protection for older adults.
Compliance	Obligation monitoring, evidence, audit, and legible accountability.
Repair maintenance and	AI-assisted diagnosis, parts, right to repair, and longer asset life.
Housing community and	Proximity, governed co-living, and adaptation for older adults.

A common feature of these industries is that the technological frontier is not always the model. It is integration with operations, regulation, and trust. A startup can build an AI demonstration for cemeteries in two weeks. It cannot simultaneously obtain access to records, standardize data, train staff, and assume responsibility. That weight is the advantage. Difficult markets protect companies willing to do the work.

Another criterion is technological timing. A commoditized industry becomes transformable when sensor costs fall, digital identity matures, regulation changes, models can process documents, or distribution moves online. The need does not have to be invented. The entrepreneur must notice when a constraint disappears. Telemedicine did not invent clinical embarrassment; it removed the waiting room. AI does not invent bureaucracy; it may reduce the cost of reading it. Cryptography does not invent inheritance; it may make digital transfer safer.

For investors, unpleasant industries have a narrative problem and a truth advantage. They do not go viral easily, but customers call. They do not promise to “change the way humanity X.” They promise that the service arrives, the document is correct, and the smell goes away. The economy needs such companies more than it needs another tool that summarizes the meeting in which the purchase of another summarization tool was approved.

The atlas does not claim that every dirty market is attractive. Some are captured, low-margin, high-risk, and populated by conservative buyers. But the founder's social discomfort is a poor filter. When everyone prefers products they would personally use in a café, sectors nobody wishes to experience firsthand remain unoptimized.

Smell, shame, and bureaucracy protect the margin. Not because people suffer elegantly, but because few founders have the patience to remain beside the problem long enough to understand it. Innovation often begins exactly where the pitch deck needs washing after the visit.

CHAPTER 11

The Great Indecent Question: Is Psychological Distress the Next Megamarket—or Merely the Next Colony?

Mental health is already an enormous market: therapy, psychiatry, pharmaceuticals, apps, coaching, wellness, sleep, mindfulness, communities, sick leave, corporate benefits, and content. Asking whether psychological distress is “the next great opportunity” may seem late, like discovering oil after someone has built the filling station. The relevant question is which part of the market will expand, who will pay, and whether growth will reduce suffering or merely make suffering easier to monetize.

Demand has three engines. The first is recognition. Stigma is declining in some settings, language is spreading, and people identify symptoms earlier generations concealed. The second is exposure. Work, loneliness, uncertainty, living costs, crises, and digital environments can amplify risk. The third is inadequate supply. Human services are expensive, unevenly distributed, and difficult to scale. When need becomes visible faster than capacity expands, technology enters the gap.

That space can produce genuine value. Triage, scheduling, telemedicine, validated digital interventions, between-session support, and clinician tools can expand access. Exercise, sleep, social connection, and work design have evidence bases capable of supporting products and services, within limits. A 2024 BMJ meta-analysis of 218 studies and 14,170 participants found reductions in depression for walking or jogging, yoga, strength training, and other forms of exercise, although evidence quality varied and exercise is not a universal substitute for care (Noetel et al., 2024). A large four-day-workweek study suggests that genuinely reduced and reorganized working time can improve well-being (Fan et al., 2025). WHO’s report on social connection identifies interventions at individual, community, and policy levels (WHO, 2025).

The market nevertheless has a structural temptation: converting social determinants into individual subscriptions. Housing is unstable, but coaching is supplied. The schedule is impossible, but breathing exercises are supplied.

Community has disappeared, but an AI companion is supplied. The clinician is unavailable, but a bot that cannot prescribe and bears no responsibility is supplied. Each product may help a little. Together they can normalize a society that outsources the cost of adaptation to the individual.

The megamarket will not consist only of treating disorders. It will include management of the subclinical state: stress, loneliness, fatigue, lack of meaning, performance anxiety, concentration difficulty, and the sense that life is perpetually behind. Commercially, this territory is ideal. The population is large, diagnostic boundaries are flexible, outcomes are subjective, and products can be sold directly without medical reimbursement. It is also the territory in which marketing can manufacture the need it claims to serve.

The wellness influencer diagnoses the audience, describes symptoms broad enough for almost anyone to recognize, and then offers the protocol, course, or supplement. The algorithm rewards certainty, not differential diagnosis. “It may be” has poor retention. “This is why you are exhausted” converts. Complex suffering is consequently reduced to cortisol, trauma, dopamine, narcissism, or a “dysregulated nervous system,” depending on the creator’s inventory.

Research is not immune to markets. Positive findings, novel effects, and spectacular technologies receive attention. Companies finance evaluations, journals prefer results, and the press promotes association into causation. The serious opportunity therefore includes an evidence infrastructure: pragmatic trials, replication, active comparators, adverse-event monitoring, and publication of negative results. In digital mental health, a product may be updated weekly while the study of its previous version takes a year. We need continuous evaluation and regulation, not an eternal seal granted to an interface that no longer exists.

Two Businesses Using the Same Vocabulary



The due-diligence question: does profit rise when the person recovers—or when the person returns?

Figure 8. The ethical line between reducing suffering and administering dependency.

Who pays? Direct consumers pay for discretion, speed, and control, but this market favors income and can exploit desperation. Employers pay for productivity and retention but may have conflicts of interest over data. Insurers pay when savings are demonstrable but may constrain access. Public systems can create equity but purchase slowly. Families pay when the person cannot. Each payer creates a different product.

The greatest opportunity may lie in coordination across payers and services. A person with depression may need a clinician, medication, activity, workplace adjustments, social support, and financial assistance. The market is fragmented into products that optimize one moment. A care operating system that coordinates, measures, and escalates could create value, but it would also centralize the most sensitive data. The architecture should be federated, consent-based, and permissioned by role. In mental health, elegant centralization can become an elegant breach.

Prevention offers a distinctive opportunity, but prevention has an economic problem: the benefit arrives later and often accrues to someone else. A school invests and the health system saves. An employer invests and the employee leaves. A city builds social space and benefits appear in health, education, and safety. Commercial models must aggregate beneficiaries or use public funding and outcome-based

contracts. Otherwise, the market will continue serving the crisis, where payment is urgent.

TABLE 9. TESTS OF A LEGITIMATE MEGAMARKET

TEST	SIGNAL AND IMPLICATION
Need	Is it real and frequent rather than principally manufactured by marketing?
Mechanism	Does the product address a plausible mechanism or merely promise “wellness”?
Evidence	Is there a measurable outcome and a relevant comparison?
Payer	Who saves when the person recovers?
Exit	Can the customer become more autonomous and less dependent?
Externalities	Does the product reduce or normalize the harmful environment?

The mental-health business that can be approached with relative confidence usually takes one of three forms. The first is administrative infrastructure: scheduling, billing, documentation, networks, and referral. The second is better distribution of human services. The third is a validated digital intervention for a precise population and outcome. The most hazardous is the general companion that promises everything while insisting it practices nothing.

Psychological distress will also become a megamarket because of AI at work. Automation will generate gains and anxiety, remove tasks and alter professional identities. In 2025, the OECD reported that 31 percent of small and medium-sized enterprises surveyed across seven countries already used generative AI, principally to improve performance and compensate for skill shortages, while training, rules, and legal preparedness remained uneven (OECD, 2025). Each wave of efficiency will create a secondary market for reskilling, transition, meaning, and psychological protection.

The opportunity is not necessarily to treat anxiety produced by automation. It may be to reduce uncertainty through transparent role design, scenario simulation, paid training, portable benefits, and internal transitions. A product that helps a company automate without needlessly destabilizing people can create value. When

sold by the same firm that recommends the layoffs, however, empathy becomes packaging for cost reduction.

The great indecent question has two answers. Yes, psychological distress is among the largest opportunities created by modernity. Need is immense, services are inadequate, technology can lower costs, and outcomes can be measured. And yes, it can become the next colony: an interior territory in which every emotion is extracted, classified, and converted into retention.

The difference will be determined by the revenue model, the standard of evidence, and the user's right to leave more autonomous than they arrived. A product that needs you to remain vulnerable is not treatment. It is an emotional plantation with minimalist design.

The Researcher, the Futurist, and the Person with a Ring Light: Three Ways to Predict Without Paying for Error

Technological prediction is an industry in which the product expires while the reputation survives. The researcher speaks in confidence intervals and loses the audience. The futurist builds scenarios and gains an audience when the story is coherent. The influencer announces certainty on Tuesday, sells the protocol on Wednesday, and deletes the post when Thursday arrives with different data. All three may possess insight. They do not all pay the same epistemic price.

Serious researchers studying mental health and technology tend to say things of infuriating moderation. Average effects of digital use are often small and heterogeneous. Causality is difficult. Screen time is a crude measure. Vulnerability, design, content, context, sleep, and compulsive patterns matter. AI may expand access and create new risks. We need longitudinal studies, active controls, adverse-event monitoring, and data protection. This kind of conclusion is correct and almost impossible to print on a mug.

Credible futurists work with scenarios, not prophecies. They ask what happens when several trends converge: the cost of AI conversation falls; models become vocal and multimodal; sensors gather behavioral data; human services remain scarce; loneliness increases. A massive market for automated companionship and support plausibly follows. It does not follow inevitably that people will abandon relationships or that therapy will be fully replaced. A scenario is an instrument of preparation, not a contract with the future.

The unqualified influencer uses a different model. A trend is selected, assigned a single cause, and converted into an identity for the audience. “Your dopamine has been destroyed.” “AI will replace every therapist.” “The phone created the entire mental-health crisis.” “Trauma is stored in the body, and this exercise releases it in ninety seconds.” Each claim may contain a real nucleus. The problem is the leap from possible mechanism to total explanation, followed by the leap from total explanation to product.

The market rewards that leap. An algorithm struggles to monetize “it depends.” It can monetize a story that supplies culprit, symptom, and ritual. Certainty is epistemic Viagra: an immediate effect, private satisfaction, and little appetite for discussing limitations. The user feels that something has been understood. The understanding may be false; its anxiolytic effect is genuine.

Researchers make mistakes too. They may overvalue statistical significance, study unrepresentative samples, use weak measures, and interpret small effects too generously. Publication favors novelty. Funding can create conflicts. When technology changes quickly, results may describe yesterday’s product. Academic authority is not immunity; it is a collection of procedures that lowers the probability of error and permits correction. The difference from the influencer is not purity. It is the infrastructure of challenge.

The dispute over social media and adolescents illustrates the point. Orben and Przybylski found small effects and criticized analytic flexibility. Twenge, Haidt, and colleagues argued that certain choices can understate harm, especially for girls and heavy users. Subsequent umbrella reviews, meta-analyses, and longitudinal studies have continued to find modest, heterogeneous effects that are more consistent for problematic use than for raw duration. The mature position is neither “nothing” nor “catastrophe.” Average risk may be small, its distribution unequal, mechanisms identifiable, and design consequential.

In AI, researchers now speak from an early evidence period. Therabot shows that a carefully adapted and supervised system can produce benefit. Studies of companionship and intensive use raise signals about dependency and loneliness. Clinical commentary warns about psychosis, sycophancy, and privacy. None of these lines justifies a final verdict. They justify investment in safety and active comparisons before distribution becomes universal.

International organizations generally predict in terms of capacity and governance. WHO calls for transforming the environments that shape mental health and strengthening community services. Its Commission on Social Connection treats isolation as a health determinant and recommends action across policy, communities, and individuals. The ILO warns that digitization, AI, and new forms of work may amplify psychosocial risk when poorly managed, while increasing flexibility and work quality when responsibly designed (WHO, 2022; WHO, 2025; ILO, 2026). These institutions do not predict the singularity. They predict that bad management will remain bad management with better software.

What can reasonably be anticipated by the early 2030s? First, AI will increasingly mediate initial contact with services through triage, information, scheduling, monitoring, and documentation. Second, validated clinical products for narrow indications will appear beside a much larger, poorly delimited wellness market. Third, AI companions will grow, particularly among young people, older adults, and isolated populations. Fourth, behavioral and vocal data will be used to detect changes in state. Fifth, conflict will intensify over who owns and uses those data. None is certain in detail; all have enough economic force behind them that ignoring them would be naïve.

Over a longer horizon, three scenarios deserve serious attention. In the augmentation scenario, AI reduces administration, expands access, supports clinicians, and facilitates human relationships. In the stratification scenario, the majority receives automated support while human care becomes a premium product. In the captivity scenario, commercial companions become emotional infrastructure and companies control the memory, personality, and availability of a simulated relationship. The actual future may contain all three, distributed by class, country, and age.

Serious futurists should be judged by mechanisms, alternative scenarios, time horizon, and history of correction. Anyone who says “this will happen” should explain what could prevent it. Anyone who says “AI will eliminate loneliness” should define loneliness, describe the substitution, and offer falsification conditions. Anyone who says “AI will destroy human connection” should explain why the telephone, automobile, migration, and television have not already completed the destruction.

Influencers may have value as cultural sensors. They notice language, anxieties, behaviors, and markets before studies are published. Forums, searches, and viral content can signal needs. But a signal is not evidence. A viral clip about “dopaminergic burnout” may indicate an exhausted population, not the existence of the clinical entity being described. The intelligent entrepreneur listens to the symptom and verifies the mechanism.

TABLE 10. HOW TO READ PREDICTIONS ABOUT THE FUTURE

SOURCE	SIGNAL AND IMPLICATION
Consensus meta-analysis and	Useful for effect size and heterogeneity; limited by study quality.

SOURCE	SIGNAL AND IMPLICATION
RCT and longitudinal study	Stronger for causality and direction; may be short or unrepresentative.
Disciplined foresight	Scenarios, mechanisms, and alternatives; not prophecy.
Clinical expertise	Detects risks and cases; may overgeneralize experience.
Influencer	Useful as a cultural sensor; weak as a causal arbiter.
Speculator	Signals where capital is flowing, not where truth resides.

The Evidence Gradient Is Not a Stairway to Infallibility

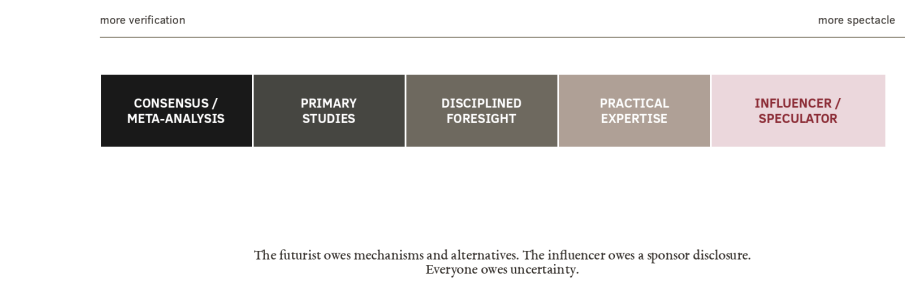


Figure 9. Predictive sources differ not only in certainty, but in their incentives to appear certain.

Financial speculators have another interest: converting prediction into valuation. They may correctly see that digital mental health, longevity, AI companions, and elder care will grow. They may be wrong about who captures the value. Large demand does not guarantee margin, and good technology does not guarantee distribution. Health is regulated, payers are fragmented, sales are slow, and outcomes matter. A chatbot with a glittering demo can lose to a company that knows how to bill an insurer and survive an audit.

This book proposes one rule of predictive hygiene. The more spectacular a claim is, and the closer it sits to the speaker’s product, the more confidence should be discounted. The more an author explains limitations, alternatives, and the evidence that would change their mind, the more attention they deserve. The rule is imperfect. Genius can sound too certain, and mediocrity can hide in nuance.

Statistically, however, a person with a ring light and a discount code is not a consensus committee.

The most useful prediction is that modernity will continue to generate new forms of discomfort and new instruments of relief faster than institutions can evaluate the relationship between them. This is an opportunity for research, enterprise, and regulation. It is also an invitation to modesty. The psychological future is not written by technology alone, but by incentives, norms, design, and the distribution of power.

Responsible prediction does not say what will happen. It says what must be measured before a seductive story becomes infrastructure.

How to Sell the Antidote Without Becoming the Poison: An Ethics Manual for People Who Still Have Payroll

Entrepreneurial ethics is often presented as a brake installed after the engine has been built. The company grows, an incident occurs, an advisory council is formed, and someone inserts the word “responsible” into a vice president’s title. In markets built around suffering, ethics cannot be a department. It is the product architecture, the revenue model, and the company’s definition of success.

The first question is simple: what happens if the product works perfectly? If the answer is that users become more autonomous and need it less often, the company must be able to survive customer success. If the answer is that users spend more time, disclose more, and grow more attached, the product may contain an internal contradiction. A loneliness service optimized for conversation with the bot is like a mobility clinic that rents the wheelchair and locks the rehabilitation room.

The revenue model is the first ethical intervention. Advertising and engagement are badly aligned with mental health. Subscription is better in some respects, but it can reward dependency and retention. Payment for outcomes may align incentives, but it requires robust measurement and may encourage selection of easy cases. Contracts with employers or insurers can expand access while introducing data and discrimination risks. No model is pure. Some merely make their conflicts visible and manageable.

The second question is which data are strictly necessary. Psychological products are tempted to collect everything: conversations, voice, typing rhythm, sleep, location, contacts, and mood. Each variable promises personalization. Together they construct a behavioral replica that the user cannot easily move or delete. Data minimization should be treated as technical performance. Local processing, encryption, separation of identity, limited retention, and control over memory can become market differentiators.

The third question is when the product must refuse. Mental-health systems cannot be designed only for the average conversation. They must recognize crisis, psychosis, abuse, suicidal risk, and situations in which uncertainty is too high. Refusal does not mean abandonment. It means escalation to appropriate resources that are geographically relevant and actually available. A button leading to an unanswered number is compliance, not safety.

The fourth question is how adverse effects are reported. Medicine has pharmacovigilance. Digital products have app-store reviews and a support channel that replies that the user's experience matters. We need post-deployment monitoring, standardized reporting, independent investigation, and the ability to withdraw or rapidly modify a system. A model update can change clinically relevant behavior without the user knowing. Version information must be part of the evidence.

The fifth question is who may audit. The company must not be the sole interpreter of its own data. Independent evaluation, published protocols, preregistered studies, and researcher access under strict safeguards are costs that build trust. In intimate markets, trust is not produced by a rounded typeface. It is produced by permitting a competent outsider to demonstrate that you were wrong.

The sixth question is whether the product changes the environment or merely adapts the person. For an employer, a mental-health platform should be able to recommend lower workload, greater control, and different management even when the sponsor would prefer meditation. For a school, a tool should be able to say that the collective norm of nighttime use is the problem, not merely the student's self-control. The antidote becomes poison when it makes the harmful environment more sustainable.

Pure altruism is not required. A product that lowers an insurer's costs, an employer's absence rate, or a family's burden can earn healthy revenue. The task is to identify the payer who benefits from genuine reduction of the problem. Sometimes the customer and beneficiary are different. That difference can finance prevention. It can also finance surveillance. The contract should state that the payer receives no individual data and cannot penalize use.

A useful principle is "productive exit." The product should design a path by which users leave with skills, relationships, documents, and control. A grief platform could facilitate groups and rituals that continue independently. An AI companion could gradually reduce its centrality and encourage human contact. An anxiety program could have an ending and a relapse-prevention plan. A navigation service

could export the history in an interoperable format. A company that makes departure easy appears to surrender an advantage. In fact, it builds reputation in a market where people fear capture.

TABLE 11. PRINCIPLES OF AN ANTIDOTE THAT DOES NOT BECOME POISON

PRINCIPLE	SIGNAL AND IMPLICATION
Success	Autonomy and functioning, not engagement.
Data	Minimization, encryption, control, and limited retention.
Escalation	Responsible refusal and genuine human access.
Evidence	Independent evaluation, versioning, and adverse-event reporting.
Revenue model	A payer aligned with reduction of the problem.
Exit	Export, completion, and relationships or skills that remain.

Another principle is the separation of persuasion from treatment. An interface may use nudges to support adherence, but it must not manipulate vulnerability. A reminder to practice an exercise may help. A notification suggesting that the bot misses you exploits attachment. An avatar that looks sad when a subscription is canceled is not empathy. It is emotional debt collection.

Companies should define prohibited categories of monetization. Mental-health data should not be used for behavioral advertising, credit, employment, or individualized pricing. Models should not optimize for intimate disclosure. Conversations should not become persuasion profiles. Some revenue must be refused; otherwise the product becomes exploitation infrastructure with wellness certificates.

In death and grief industries, ethics means transparent prices, absence of pressure, consent for digital memorials, and protection of the posthumous image. In sexual health, it means clinical assessment, privacy, and marketing that does not manufacture shame. In elder care, it means that a sensor does not replace the visit and that data do not turn an older person into an object of family control. In security, it means fear is not amplified for an upsell. The principles remain the same even when the industry smells different.

Internal governance must be connected to power. An ethics committee without veto authority is theater. Safety teams must be able to halt a launch, and risk metrics

must reach the board. Executive compensation can incorporate safety and autonomy outcomes. Whistleblowers must be protected. These measures sound corporate and tedious. Real ethics is largely boredom with authority.

The founder may ask whether all this will slow the company while an unscrupulous competitor grows. Probably. Sensitive markets, however, tend to receive regulation after scandal, and companies that built the infrastructure may survive. More importantly, speed does not justify turning the customer into material. Sometimes the first-mover advantage consists of reaching the courtroom first.

Selling the antidote without becoming the poison requires accepting an unpopular truth: not every engagement is value, not every personalization is care, and not every market should be served to the limit of what is technically possible. The good company is not the one that avoids profit. It is the one that profits from reducing the problem and can demonstrate the difference.

Ethics is not the opposite of entrepreneurship. In unpleasant markets, it is the technology through which trust becomes scalable.

Escaping Modernity Without Losing the Dentist: An Architecture for Lives, Organizations, and Markets That Do Not Demand Total Nervous Sacrifice

Total escape from modernity is a luxury product sold by people who retain the internet for marketing. Retreats into nature, detox, minimalism, and “ancestral” living may help individuals, but they often depend on capital, health, and the very infrastructure they criticize. The ambulance remains modern even when the retreat is lit by torches. The realistic solution is not abandonment but selection: preserve the advantages and reject costs that arise from design rather than physical law.

At the individual level, research supports things of magnificent unoriginality: sleep, movement, relationships, meaningful activity, treatment when needed, and limits on harmful exposures. Their banality does not make them easy. The environment has been designed to compete with them. Exercise has evidence for reducing depressive symptoms, although access, severity, and preference matter (Noetel et al., 2024). Reducing social-media use may produce small or selective benefits, but it will not cure loneliness if the recovered time remains empty. Sleep hygiene can help; it cannot negotiate the night shift.

Architecture is a better strategy than willpower. The phone does not enter the bedroom. Notifications are off by default. Networks are visited in windows rather than allowed to become the background. Friendship receives recurring appointments, not merely good intentions. Movement is built into transport or routine. Work has a visible end. These are environmental changes that reduce decision load. Self-control is a resource; design can stop taxing it every minute.

At the family level, collective norms are more effective than unilateral prohibition. A phone-free meal works better when adults do not answer email under the banner of responsibility. Adolescents need autonomy and limits, education about platform design, and genuine social alternatives. Confiscating the

phone without providing transport, space, and friends is removing the bridge after the city has demolished the square.

The modern family also needs care infrastructure. Parents, children, and older adults depend on coordination, and the burden falls disproportionately on women. Services that simplify scheduling, documents, transport, and communication can release time for relationship. Technology proves its value when care becomes more human, not when every family member becomes a user with a dashboard.

At the community level, connection cannot be delivered exclusively through content. It requires places, repetition, roles, and a low cost of participation. Libraries, parks, clubs, sport, workshops, religious communities, neighborhood centers, cooperatives, and recurring events are social-health infrastructure. WHO's 2025 report treats social connection as a determinant of health and recommends scalable action through communities and policy (WHO, 2025). Enterprise can contribute hybrid models, spaces, coordination, and matching, but it should optimize the meeting, not the feed about the meeting.

A promising market is "social fitness": services that help people form and maintain relationships through repeated activities, small groups, and accountability. The risk is converting friendship into performance and score. The product should recede into the background. When members begin messaging one another directly and no longer need the platform, it should record success rather than leakage.

At the organizational level, escape means control, predictability, realistic volume, fairness, and the right to disconnect. WHO guidance recommends intervention in working conditions, manager training, and return-to-work support rather than individual tools alone (WHO, 2022b). A large study of the four-day workweek indicates that genuinely reduced working time, supported by reorganization, can improve burnout and health (Fan et al., 2025). The lesson is not that Friday must be declared holy. It is that productivity can be converted into recovery.

AI can help organizations reduce administrative work and distribute tasks more intelligently. It can also intensify pace, expand surveillance, and remove autonomy. The difference must be negotiated. Employees need participation in implementation, transparency about evaluation, a right to contest decisions, and protection against the use of psychological data. A model should not label someone "resilient" or "flight risk" without consequences and appeal.

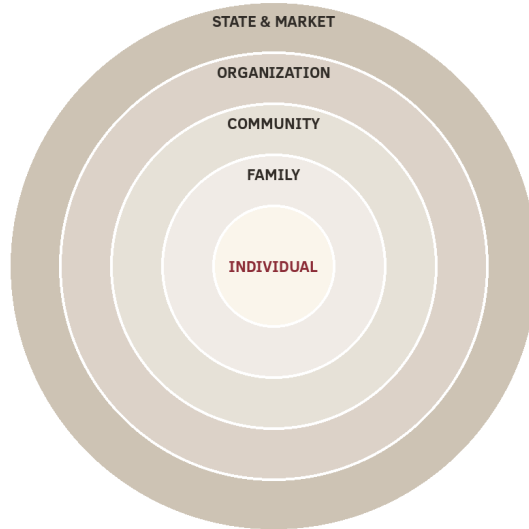
At the market level, escape requires models that do not sell the user's attention. Subscription, cooperatives, public digital services, interoperability, and outcome-based payment are alternatives. None works in every category. What

matters is that the product has a legible customer and does not turn vulnerability into a secondary raw material. When the service is “free,” the person is not always the product. Sometimes the prediction about the person is the product.

At the urban level, housing, transport, and public space are mental-health interventions. Cities can reduce commuting, noise, isolation, and insecurity through proximity, nature, accessibility, and places to meet. A park bench does not cure depression, but a society without benches, sidewalks, or public toilets communicates who has the right to remain. Relational design begins with permission to sit without purchasing.

At the policy level, we need standards for design aimed at minors, researcher access to data, algorithmic transparency, health-data protection, the right to disconnect, occupational health, and community services. Regulation should not freeze innovation, but neither should it treat the population as a study without consent. When a platform has billions of users, “move fast” is unelected public policy.

Escape Does Not Begin with a Single Habit



Personal protection without institutional change becomes another unpaid job.

Figure 10. Psychological protection is multilevel: an individual cannot compensate alone for an environment designed against them.

A functional escape has five mutually supporting layers. Individuals configure their environment and seek care. Families create norms and share care. Communities provide belonging and places. Organizations reduce risk at the source. The state establishes rights, infrastructure, and incentives. Asking only the individual to become resilient is like asking the tenant to meditate during the fire. Asking only the state to solve everything ignores the capacity of communities and products. The architecture works when the layers do not sabotage one another.

For the entrepreneur, this architecture offers a rule for selecting opportunities. Look for points where benefits are distributed but coordination is absent. Look for friction between the individual and the system, not merely the person’s internal state. Look for the payer who saves when the person recovers. Look for technologies that reduce the need for attention rather than colonizing it. Look for processes in which modest integration produces dignity.

Products can combine modern advantages with older functions. A neighborhood platform can coordinate care and lending without a public feed. A workplace service can use AI to convert productivity into shorter weeks. A companion can prepare someone for a conversation with a clinician and then withdraw. Intergenerational housing can use sensors for safety and rules for reciprocity. A funeral service can digitize administration while protecting ritual from excessive efficiency.

Escape also requires tolerance for normal discomfort. Not every sadness must be optimized, not every boredom filled, and not every conflict mediated by a system. The capacity to bear frustration, uncertainty, and silence is protection against products with immediate effects. The psychological Viagra market weakens when people can postpone the dose.

This does not mean glorifying suffering. Treatable pain should be treated. Disorders require care. Isolation should be reduced. A good life, however, is not a life without negative emotion. It is one in which emotion has context, support, and proportion. Modernity has sometimes confused comfort with the good and stimulation with life. Escape means restoring the distinction.

TABLE 12. THE ARCHITECTURE OF ESCAPE

LEVEL	SIGNAL AND IMPLICATION
Individual and family	Sleep, movement, limits, collective norms, and care.
Community	Places, repetition, roles, and reciprocity.

LEVEL	SIGNAL AND IMPLICATION
Organization	Control, realistic volume, predictability, and the right to disconnect.
Market	Alternatives to advertising, interoperability, and outcome-based payment.
State and city	Rights, services, public space, housing, and standards.

We shall keep the dentist, the antibiotic, the map, the video call, the global library, and the AI that helps us understand a document. We shall refuse, as far as possible, the notification that treats attention as abandoned property, the workplace that converts every efficiency gain into volume, the companion that needs us to remain alone, and the market that sells shame back to us as a subscription.

There is no single exit. There are millions of design decisions, contracts, norms, and products. This is bad news for anyone seeking a pill. It is good news for the entrepreneur seeking a real market.

Modernity is not a train from which we can disembark. It is a city under construction. Some sell windows, others alarms, others bottled air. The uncomfortable opportunity is to build rooms in which the nervous system does not need permission to sit down.

EPILOGUE

When the sarcasm ends, the bill remains

We began with Aspirin, Viagra, and coffins because indecent metaphors tell the truth faster than institutional language. Pain, desire, and death are reliable markets not because people are consumers, but because they are vulnerable bodies living under a deadline. Modernity has made that vulnerability less fatal in many respects and easier to measure, compare, and monetize in others.

There is no evidence that the past was a psychological paradise. There is evidence that the forms of suffering and the infrastructures of meaning changed. Small communities could protect and constrain. Agriculture brought stability and hierarchy. The ancient city brought culture and comparison. The factory disciplined time. The century of comfort reduced pain and industrialized desire. The network made status permanently visible. AI is beginning to make relationship available without the other person.

The diagnosis is not that technology is destroying us. It is that narrow optimization produces psychological externalities, and the market is intelligent enough to sell treatments for them. Sometimes the same company produces both. Sometimes the investment portfolio provides the integration.

The best entrepreneurial opportunities are often unpleasant: care, death, sex, pain, waste, security, compliance, fraud, maintenance, and coordination. In these domains, the problem need not be invented, the customer knows it exists, and technology can remove friction. They nevertheless demand operations, trust, and patience. They are not interface games. They are services in which an error enters someone's life.

Psychological discomfort is unquestionably a megamarket. It will attract clinicians, founders, insurers, employers, platforms, influencers, and speculators. Some will expand access and reduce burden. Others will convert loneliness into retention, anxiety into a segment, and grief into a funnel. The difference will not be visible in the interface's empathic tone. It will be visible in the revenue model, right of exit, data protection, long-term effects, and the company's willingness to recommend a solution that makes the company less necessary.

This book was written as an experiment with AI because the subject cannot be separated from the instrument. A language model can synthesize dozens of studies, construct metaphors, and help build a voice. It cannot assume moral responsibility for what the reader, the company, or society does with them. Responsibility remains human, which is comforting only until one recalls the record.

Escape from modernity is not refuge. It is design. It means converting productivity into time, connectivity into relationship, data into control for the person, and AI into support that strengthens agency. It means that the entrepreneur can treat suffering as a signal without treating the human being as a deposit to be mined.

When the sarcasm ends, the bill remains. Someone pays it in money, time, sleep, relationships, or years of life. A good business is not one that moves the bill far enough away to keep it out of the quarter. It is one that reduces the bill.

Aspirin should relieve. Viagra should restore function without manufacturing shame. The coffin should preserve the dignity of the person who can no longer negotiate. And technology, if it wishes to be called progress, should leave the human being with more freedom than it found.

The rest is merely engagement.

SELECTED BIBLIOGRAPHY AND NOTES FOR ORIENTATION

Selected Bibliography and Notes for Orientation

- Ansari, S., Iqbal, N., Azeem, A., & Danyal, K. (2024). Improving well-being through digital detoxification among social media users: A systematic review and meta-analysis. *Cyberpsychology, Behavior, and Social Networking*, 27(11), 753–770. <https://doi.org/10.1089/cyber.2023.0742>
- Berg, J., & Johnston, H. (2025). AI in human resource management: The limits of empiricism (ILO Working Paper No. 154). International Labour Organization. <https://doi.org/10.54394/NMSH7611>
- Blasco, S., Rochut, J., & Rouland, B. (2025). Displaced or depressed? Working in automatable jobs and mental health. *Industrial Relations: A Journal of Economy and Society*, 64(1), 40–76. <https://doi.org/10.1111/irel.12356>
- Calvert, E., Cipriani, M., Dwyer, B., et al. (2025). Social media detox and youth mental health. *JAMA Network Open*, 8(11), e2545245. <https://doi.org/10.1001/jamanetworkopen.2025.45245>
- Condorelli, R. (2016). Social complexity, modernity and suicide: An assessment of Durkheim's Suicide from the perspective of a non-linear analysis of complex social systems. SpringerPlus, 5, 374.
- Dahl, T., Foldnes, N., Grønneberg, S., et al. (2026). Why social media research has failed policy-makers. *Nature Human Behaviour*, 10, 1007–1008. <https://doi.org/10.1038/s41562-026-02482-9>
- Dohnány, S., Kurth-Nelson, Z., Spens, E., et al. (2026). Technological folie à deux: Feedback loops between AI chatbots and mental health. *Nature Mental Health*, 4, 336–345. <https://doi.org/10.1038/s44220-026-00595-8>. Conceptual perspective, not a causal clinical trial.
- Dong, X., et al. (2025). A meta-analysis of artificial intelligence technologies use and loneliness: Examining the influence of physical embodiment, age differences, and effect direction. Meta-analysis of 47 studies from 21 publications. PubMed PMID: 39905934.
- Durkheim, É. (1897). *Le suicide: Étude de sociologie*. Félix Alcan.
- Engels, F. (1845). *The condition of the working class in England*. Otto Wigand.
- Evans, K., McGrath, J., & Milns, R. (2003). Searching for schizophrenia in ancient Greek and Roman literature: A systematic review. *Acta Psychiatrica Scandinavica*, 107(5), 323–330.
- Fan, W., Schor, J. B., Kelly, O., et al. (2025). Work time reduction via a 4-day workweek finds improvements in workers' well-being. *Nature Human Behaviour*, 9, 2153–2168. <https://doi.org/10.1038/s41562-025-02259-6>

- Fang, C. M., Liu, A. R., Danry, V., et al. (2025). How AI and human behaviors shape psychosocial effects of chatbot use: A longitudinal randomized controlled study. *arXiv:2503.17473*. Preprint.
- Fassi, L., Thomas, K., Parry, D. A., Leyland-Craggs, A., Ford, T. J., & Orben, A. (2024). Social media use and internalizing symptoms in clinical and community adolescent samples: A systematic review and meta-analysis. *JAMA Pediatrics*, 178(8), 814–822. <https://doi.org/10.1001/jamapediatrics.2024.2078>
- Fiore, M., Arena, D., Crisafi, V., et al. (2025). Digital devices use and sleep in adolescents: An umbrella review. *International Journal of Environmental Research and Public Health*, 22(10), 1517. <https://doi.org/10.3390/ijerph22101517>
- Folk, D., & Dunn, E. (2026). How does turning to AI for companionship predict loneliness and vice versa? *Psychological Science*, 37(4), 276–286. <https://doi.org/10.1177/09567976261427747>
- Heinz, A., Deserno, L., & Reininghaus, U. (2013). Urbanicity, social adversity and psychosis. *World Psychiatry*, 12(3), 187–197. <https://doi.org/10.1002/wps.20056>
- Heinz, M. V., Mackin, D. M., Trudeau, B. M., et al. (2025). Randomized trial of a generative AI chatbot for mental health treatment. *NEJM AI*, 2(4). <https://doi.org/10.1056/AIoa2400802>
- International Labour Organization. (2026). The psychosocial working environment: Global developments and pathways for action. International Labour Organization. <https://doi.org/10.54394/00033223>
- Teague, S., Somoray, K., Shatte, A., et al. (2026). Digital media use and child health and development: A systematic review and meta-analysis. *JAMA Pediatrics*, 180(5), 510–517. <https://doi.org/10.1001/jamapediatrics.2026.0085>
- National Academies of Sciences, Engineering, and Medicine. (2024). Social media and adolescent health. The National Academies Press.
- Noetel, M., Sanders, T., Gallardo-Gómez, D., et al. (2024). Effect of exercise for depression: Systematic review and network meta-analysis of randomised controlled trials. *BMJ*, 384, e075847. <https://doi.org/10.1136/bmj-2023-075847>
- Odgers, C. L., & Jensen, M. R. (2020). Annual research review: Adolescent mental health in the digital age—Facts, fears, and future directions. *Journal of Child Psychology and Psychiatry*, 61(3), 336–348.
- OECD. (2025). Generative AI and the SME workforce: New survey evidence. OECD Publishing. <https://doi.org/10.1787/2d08b99d-en>
- OpenAI & MIT Media Lab. (2025). Early methods for studying affective use and emotional well-being on ChatGPT. Research report and associated experimental study; not peer reviewed at publication.
- Orben, A., & Przybylski, A. K. (2019). The association between adolescent well-being and digital technology use. *Nature Human Behaviour*, 3, 173–182. <https://doi.org/10.1038/s41562-018-0506-1>

- Ramadhan, R. N., Rampengan, D. D., Yumnaniha, D. A., et al. (2024). Impacts of digital social media detox for mental health: A systematic review and meta-analysis. *Narra X*, 4(2), e786. <https://doi.org/10.52225/narra.v4i2.786>
- Sharma, A., Lin, I. W., Miner, A. S., Atkins, D. C., & Althoff, T. (2023). Human–AI collaboration enables more empathic conversations in text-based peer-to-peer mental health support. *Nature Machine Intelligence*, 5, 46–57.
- Sohn, J.-S., Ha, B.-G., Park, S., et al. (2026). Systematic review and meta analysis of chatbots in the management of depressive and anxiety symptoms. *npj Digital Medicine*, 9, 377. <https://doi.org/10.1038/s41746-026-02566-w>
- Smith, D. H., & Hackathorn, L. (1982). Some social and psychological factors related to suicide in primitive societies: A cross-cultural comparative study. *Suicide and Life-Threatening Behavior*, 12(4), 195–211. <https://doi.org/10.1111/j.1943-278X.1982.tb00708.x>. The source title uses historical terminology; this edition otherwise prefers “small-scale societies.”
- Stack, S. (1993). The effect of modernization on suicide in Finland: 1800–1984. *Sociological Perspectives*, 36(2), 137–148. <https://doi.org/10.2307/1389426>
- Thompson, E. P. (1967). Time, work-discipline, and industrial capitalism. *Past & Present*, 38, 56–97.
- Tousignant, M. (1998). Suicide in small-scale societies. *Transcultural Psychiatry*, 35(2), 291–306. <https://doi.org/10.1177/136346159803500207>
- U.S. Surgeon General. (2023). Social media and youth mental health: The U.S. Surgeon General’s advisory. U.S. Department of Health and Human Services.
- Valkenburg, P. M., Meier, A., & Beyens, I. (2022). Social media use and its impact on adolescent mental health: An umbrella review of the evidence. *Current Opinion in Psychology*, 44, 58–68. <https://doi.org/10.1016/j.copsyc.2021.08.017>
- Vassos, E., Pedersen, C. B., Murray, R. M., Collier, D. A., & Lewis, C. M. (2012). Meta-analysis of the association of urbanicity with schizophrenia. *Schizophrenia Bulletin*, 38(6), 1118–1123. <https://doi.org/10.1093/schbul/sbs096>
- World Health Organization. (2022). World mental health report: Transforming mental health for all. World Health Organization.
- World Health Organization. (2022b). Guidelines on mental health at work. World Health Organization.
- World Health Organization. (2022c). Mental health and COVID-19: Early evidence of the pandemic’s impact. Scientific brief and associated communications on the global increase in anxiety and depression during the pandemic’s first year.
- World Health Organization. (2025). From loneliness to social connection: Charting a path to healthier societies. Report of the WHO Commission on Social Connection. World Health Organization.

Xiao, Y., Meng, Y., Mann, J. J., et al. (2025). Addictive screen use trajectories and suicidal behaviors, suicidal ideation, and mental health in U.S. youths. *JAMA*, 334(3). <https://doi.org/10.1001/jama.2025.7829>

Final methodological note. This bibliography is selective rather than exhaustive. Sources were chosen to combine consensus reports, meta-analyses, longitudinal studies, randomized trials, and historical scholarship. Preprints and conceptual perspectives are identified as such and are not treated as equivalent to replicated clinical evidence. The literature reflected in this edition was available through July 2026.